



Safeguarding & Protection Policy Suite

Britannia Youth Organisation CIC is a Community Interest Company registered in England and Wales.
Company No. 12515346 | Registered Address: 36 St Joseph's Rd, Ward End, Birmingham, B8 2JU

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Emergency Contacts:

DSL: Hassan Kingsley (07597 874 222)

Deputy DSL: To be appointed

Emergency: 999



Core Policy Framework



Safeguarding Policy Framework

Part of the BYO Safeguarding and Protection Policy Suite

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Introduction

This Safeguarding Policy Framework serves as the umbrella document for the Britannia Youth Organisation (BYO) comprehensive safeguarding framework. It establishes the overarching principles and structure that connects all our safeguarding policies and procedures.

At BYO, our mission is to empower young people by providing essential support and resources that enhance both their mental and physical well-being. We deliver a diverse range of high-impact programmes designed to address the mental health, emotional resilience, and physical well-being of young people. As an organisation embedded in and committed to the local community and its needs, we will from time to time deliver a range of services to the wider community including older people. We recognise that through our young people's services which may include over 18's, and occasional projects to the wider community, we have a duty to consider our safeguarding responsibilities to both children and adults at risk of abuse.

All **staff** and **volunteers** should familiarise themselves with this policy framework and all related procedures. Failure to follow this policy could result in disciplinary proceedings and dismissal.

1. Purpose & Application

BYO are committed to fostering a safe, inclusive, and nurturing environment where the welfare of children and adults at risk is promoted at all times. We believe that any form of abuse to children or adults is unacceptable. We take our responsibility to safeguard children and adults seriously and our safeguarding framework is designed to help us integrate safeguarding into how we safeguard those we come into contact with effectively by how we:

- Plan and deliver our services
- Recruit, induct, train and set expectations of our **staff** and **volunteers**
- Ensure that service users, parents, our partners and members of the public know how to tell us if they have concerns
- Recognise abuse; respond to and manage any safeguarding concerns or allegations
- Keep listening - to our service users, **staff** and **volunteers**, our partners and wider community so we can learn how to improve

The purpose of this framework is therefore to ensure that all **staff** and **volunteers** at BYO, including **Directors**, recognise the importance of safeguarding; know what their specific responsibilities are in contributing to a safe culture and are supported to understand how to keep people safe. We want the young people and adults we work with to feel safe and know who to tell if they have concerns, being confident that they will be listened to, taken seriously and that appropriate action will be taken. This framework sets out for everyone in a transparent way, our safeguarding responsibilities, how we will meet them and the principles that underpin how we work.

This policy applies to all our **staff** and **volunteers** (including contractors, student placements, temporary or agency **staff**), and visitors.

2. Legislation and Keeping the Policy Up to Date

This framework reflects current legislation and best practice including the Children Act 1989, Children Act 2004, Care Act 2014, Protection of Freedoms Act 2012, Equality Act 2010, Human Rights Act 1998, Mental Capacity Act 2005, Safeguarding Vulnerable Groups Act 2006, Working Together to Safeguard Children 2024, and relevant statutory guidance.

Our Safeguarding Policy Framework is reviewed annually to ensure it is in line with current legislation. We may also review and update the framework from time to time in response to incidents, material changes in law or our operations.

3. Master Definitions Glossary

The following definitions apply consistently across all BYO safeguarding policies and procedures:

Children

A **child** is anyone under the age of 18.

Article 19 of the UN Convention on the Rights of the Child states that “every child should be protected from abuse”.

Working Together to Safeguard Children 2024 states that everyone who comes into contact with children and families has a role to play. It describes safeguarding and promoting the welfare of children as:

- Protecting children from maltreatment
- Preventing impairment of children’s health or development
- Ensuring that children grow up in circumstances consistent with the provision of safe and effective care
- Taking action to enable all children to have the best outcomes

Adults at Risk

An “**adult at risk**” is someone who is aged 18 or over who:

- Has care and support needs and
- Is experiencing, or is at risk of, abuse or neglect; and
- As a result of those care and support needs is unable to protect themselves from either the risk of, or the experience of, abuse or neglect.

The Care Act 2014 states that “adult **safeguarding** is working with adults with care and support needs to keep them safe from abuse or neglect.” Where adults may lack the mental capacity to make specific decisions about their safety or welfare, we will follow the principles and requirements of the Mental Capacity Act 2005, ensuring that any decisions made are in their best interests and involve them as much as possible.

Key Personnel Definitions

- **Staff:** All employed personnel including full-time, part-time, contractors, student placements, temporary or agency workers
- **Volunteers:** All unpaid personnel who provide services to BYO including regular and occasional volunteers
- **Directors:** Senior leadership team responsible for operational oversight
- **Governor:** Strategic leader with overall organisational responsibility
- **DSL: Designated Safeguarding Lead** - person with primary responsibility for safeguarding matters
- **DDSL: Deputy Designated Safeguarding Lead** - designated alternative to the **DSL**

Safeguarding

Safeguarding means protecting people’s health, well-being and human rights, and enabling them to live free from harm, abuse and neglect. This includes both proactive measures to prevent harm and reactive responses to concerns or incidents.

4. Responsibilities

BYO is a Community Interest Company and is not regulated by the Charity Commission for England and Wales (Charity Commission). However, for the purpose of adopting best practice, BYO is guided by Charity Commission expectations in relation to safeguarding.

Governor

The **Governor**, as strategic leader of BYO, has overall responsibility for safeguarding and will ensure it is central to BYO's culture, strategy and delivery.

The **Governor** will ensure that BYO:

- Establish safeguarding policies and procedures which fit with those of the local authority and which all BYO **staff** and **volunteers** follow
- Make sure all **staff** and **volunteers** receive regular training on safeguarding
- Manage concerns, complaints, whistleblowing and allegations effectively
- Have clear policies as to when DBS checks are required, assessing the level of check needed and how the sensitive information is handled in accordance with the Safeguarding Vulnerable Groups Act 2006
- Issues are reported to external bodies, where appropriate such as DBS, police, local authority

Director

Directors are responsible for operational matters. They will:

- Work with the **Governor** to maintain this framework through planning, implementing, measuring and reviewing performance on a corporate level
- Oversee the effective implementation of the framework
- Ensure **staff** and **volunteers** understand their responsibilities in creating a safe culture
- Lead on developing and maintaining a culture of openness, where everyone feels able to speak up and voice concerns
- Ensure that any concerns or allegations are promptly investigated and reported
- Undertake regular safeguarding training

Managers

Those **staff** with line management responsibility are accountable for ensuring the framework is implemented in the areas that they oversee. They should:

- Ensure the framework is communicated in a way that is understood
- Check that **staff** and **volunteers** are clear about their safeguarding responsibilities
- As a role model, promote a safe culture at all times, informing, instructing and supervising staff and volunteers
- Ensure all **staff** and **volunteers** are recruited, inducted and trained in line with this framework
- Undertake relevant safeguarding training for themselves at least every 3 years
- Ensure all incidents relating to safeguarding are reported to a **Designated Safeguarding Lead (DSL)**
- Report safeguarding issues which they cannot resolve to the **Directors**
- Ensure that any concerns or allegations are promptly investigated and reported

All Staff and Volunteers

Safeguarding is everyone's responsibility. All **staff** and **volunteers** should:

- Conduct themselves in line with the Code of Conduct at all times
- Follow the safeguarding arrangements set out in this framework and related policies
- Report all safeguarding issues to the **DSL** without delay
- Report concerns about the behaviour of **staff** or **volunteers** to the **DSL** without delay. Where the concern relates to the conduct of the **DSL**, it should be reported to **DDSL**

See also: [Whistleblowing Policy] - If you feel unable to report concerns directly through normal channels, alternative reporting options are available.

Designated Safeguarding Lead (DSL)

The **DSL** has overall responsibility for safeguarding matters in BYO. They will:

- Provide safeguarding advice to BYO
- Promote a safe culture and challenge behaviour that undermines safeguarding
- Receive concerns from staff, volunteers and others
- Make referrals, as appropriate to the local authority and ensure other external reporting takes place, where appropriate
- Ensure that any concerns or allegations are promptly investigated and actions taken
- Keep records of concerns, allegations and other safeguarding related issues securely
- Monitor how effectively this framework is being implemented, raising any issues with the Board
- Note any issues that present a risk to the organisation and report these to the Board
- Ensure DBS checks and training for all staff and volunteers is up to date and that up to date records are held centrally
- Complete appropriate safeguarding training at least every 2 years

5. Safeguarding Commitments

BYO recognises that a significant part of effective **safeguarding** is to have in place preventative measures. Whilst we can never fully eradicate the risk of abuse and harm, we should do what we can to reduce its likelihood or reduce its escalation by acting fast. BYO therefore makes the following commitments in order to support the full and effective implementation of **safeguarding**.

Recruitment

We practice safer recruitment to ensure that we recruit people who are safe to work with **children** and **adults at risk**. These measures include:

- Applying more robust measures to roles that involve contact with **children** or **adults at risk**
- Seeking the highest level of DBS check for the role, subject to legal eligibility under the Safeguarding Vulnerable Groups Act 2006
- Seeking a renewed DBS check at least every 3 years. A renewed check will also be triggered where someone changes role; is the subject of a safeguarding related investigation or where the **DSL** reasonably believes it is appropriate
- Ensuring a full induction proportionate to the role
- Ensuring ongoing supervision and line management

See also: *[Recruitment, Selection and Onboarding Policy]* and *[Vetting and DBS Policy]* for detailed procedures.

Training

- Ensure safeguarding training appropriate to the role is completed within 3 months of starting the role
- Training is refreshed at least every 3 years, or sooner depending on risks and level of responsibility

Conflict of Interest

As a community-based organisation with people who may hold multiple roles, BYO recognises the importance of managing potential conflicts of interest. We address this through:

- Clear role definitions and accountability structures to ensure safeguarding responsibilities are transparent
- Appropriate oversight and supervision arrangements where individuals hold multiple roles
- Clear protocols for managing situations where **staff** or **volunteers** have personal relationships with service users
- Regular review of governance arrangements to ensure independence in safeguarding decision-making
- Clear escalation routes that bypass potential conflicts of interest where necessary

6. Risk Assessment and Diversity

BYO believe that everyone should be safeguarded from abuse or harm and should never be discriminated against on the basis of age, disability, gender reassignment, marriage and civil partnership, pregnancy and maternity, race, religion or belief, sex, sexual orientation, as protected under the Equality Act 2010. We recognise that everyone has fundamental human rights as enshrined in the Human Rights Act 1998, including the right to life, liberty and security of person.

BYO celebrates diversity and promotes equality through its Equality, Diversity and Inclusion Policy. We recognise that some individuals and communities may be more at risk of harm. We deliver services in an area of high deprivation which can increase the risk of some harms. For example, many **children** and young people we work with may be affected by neglect, domestic violence, or discrimination on the basis of race. Local statistics indicate that school attendance rates are below the national average and **children** experience exclusion due to anti-social behaviour. We are mindful of the issues facing the **children** and young people and adults we work with and carry out risk assessments for all our activities to ensure we address both generic and specifically known risks.

See also: *[Equality, Diversity and Inclusion Policy]* and *[Health and Safety Policy]* for comprehensive risk assessment procedures.

7. Data, Consent and Information Sharing

All information is processed and managed in line with data protection legislation and our Data Protection Policy. This includes ensuring we have a privacy statement when requesting personal and/or sensitive information; obtaining parental consent for activities, taking and using of images and ensuring that information is held securely.

BYO recognises that data protection is not a barrier to **safeguarding children** and adults and we will share information appropriately where it is for the purpose of keeping people safe. We understand that working in partnership with other agencies is essential to keeping people safe. Wherever possible, we will seek consent before so doing, unless it would place people at greater risk.

See also: *[Data Protection Policy]* for detailed information handling procedures.

8. Digital Safeguarding

BYO recognises the importance of keeping people safe in digital environments. Our approach includes:

Use of Mobile Phones and Devices:

- **Staff** are issued with work mobile phones for contact with parents and other adults during working hours
- **Volunteers** using personal phones must only use them for communication between **staff/volunteers**, not directly with parents/**children**, unless covered by a clear written agreement
- All digital communication must be professional and appropriate

Communication with Children:

- Direct email or phone contact with **children** under 16 should be avoided
- Any necessary email contact should be directed to parents/carers or copy them in
- Clear protocols exist for responding to unexpected contact from **children**

Record Keeping:

- All texts, emails, and calls with parents/adults or **children** are recorded appropriately
- Digital communication logs are maintained securely

Social Media and Online Platforms:

- **Staff** and **volunteers** must not post images or information about BYO work on personal social media
- Friend requests or social media connections with service users are not permitted
- All social media use follows our Social Media Policy

See also: *[Digital Safeguarding Policy]* for comprehensive digital safety procedures.

9. Joint Working

BYO works jointly with other organisations through venue partnerships, collaborative events, and shared programmes. Before entering into any joint enterprise we will carry out due diligence checks to ensure that the organisation or individual does not pose a reputational or **safeguarding** risk.

Joint working arrangements will always be recorded in writing and signed by relevant parties. It will include key details including **safeguarding** protocols and who will lead on **safeguarding** matters. Regardless of which organisation takes the lead role for **safeguarding**, BYO will also retain the right to refer matters to the local authority where they believe it is appropriate and where the lead organisation fails to do so.

10. Reporting Concerns and Alternative Pathways

All safeguarding concerns should be reported immediately to the DSL. However, we recognise that there may be circumstances where individuals feel unable to report concerns through normal channels.

Alternative reporting options include:

- Reporting to the DDSL if concerns relate to the DSL
- Using our Whistleblowing Policy procedures for serious concerns about organisational practices
- Accessing our Complaints Policy for service-related concerns
- Direct contact with external authorities where immediate risk is present

See also: *[Whistleblowing Policy]* and *[Complaints Policy]* for detailed alternative reporting procedures.

11. Key Contacts

Designated Safeguarding Lead: Hassan Alexander Kingsley

Mobile: 07597 874 222

Email: safeguarding@britanniayo.com

Deputy Designated Safeguarding Lead: To be appointed

Email: safeguarding@britanniayo.com

Emergency Contacts:

- **Police:** 999
- **Children's Services:** [Insert local authority contact]
- **Adult Social Services:** [Insert local authority contact]
- **NSPCC Helpline:** 0808 800 5000

This framework should be read as the foundation document alongside all other BYO safeguarding policies and procedures. For any queries about this framework, please contact the Designated Safeguarding Lead.

This policy has been approved by Hassan Kingsley, the Governor, and will be reviewed annually or following any significant incident or change in legislation.

Reviewed by:



Hassan Kingsley

Governor

Date: 28/06/2026

Next Review Date: 30/06/2027

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Child Safeguarding Procedures

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**Designated
Safeguarding Lead**

Hassan Kingsley

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**Deputy Designated
Safeguarding Lead**

To be appointed

☎

✉

Introduction

Safeguarding children and promoting their welfare is of paramount importance at BYO. These procedures should be read in conjunction with our **Safeguarding Policy Framework**. When planning and delivering any activities with children and young people, staff and volunteers must follow the BYO Safeguarding Policy Framework and our **Standing Operating Procedure for CYP Events**.

BYO delivers services for young people which includes both children (under 18) and young adults (18-24). We strive to have appropriate safeguards in place for young people and adults at all times. Where there is a safeguarding concern, staff and volunteers will follow the appropriate safeguarding procedures (child protection or adult safeguarding), as needed.

See Also: For guidance on planning and delivering activities, refer to our **Standing Operating Procedure for CYP Events**, **Health and Safety Policy**, and **Offsite Visits Policy**.

1. Purpose & Application

These procedures are for staff and volunteers of BYO and set out what you are expected to do when you have a safeguarding concern about a child or young person under 18 years of age. This includes where:

- A child or young person tells you they are being abused or are at risk of abuse
- Someone tells you about a child or young person who is being abused or is at risk of abuse
- You see or hear something that indicates a child may be at risk of or experiencing abuse

Different Procedures Apply For:

Concerns about staff or volunteer behaviour: Report to DSL/DDSL immediately - **Managing Allegations about Staff and Volunteers Policy** applies

Concerns about harmful behaviour by children/young people: Report to DSL/DDSL - **Managing Allegations of Peer-to-Peer Abuse Policy** applies

Safeguarding concerns about adults (18+): Follow **Adult Safeguarding Procedures**

Whistleblowing Route: If you feel unable to report concerns directly through these channels, see our **Whistleblowing Policy** for alternative reporting options.

2. What is Abuse

There are many kinds of abuse and ways in which children and young people can experience harm, some of which are listed below:

- Physical
- Emotional
- Neglect
- Child Sexual Abuse
- Child Sexual Exploitation
- Child Trafficking and Slavery
- Domestic Violence
- Female Genital Mutilation
- Grooming
- Bullying and Cyber Bullying
- Sextortion
- Radicalisation
- County Lines
- Harmful Sexual Behaviour

(See Appendix 1 for detailed descriptions)

Abuse may be deliberate. It can also be the result of ignorance or lack of training, knowledge or understanding but this does not make it any less harmful and it must be acted on in line with BYO Safeguarding Policy and Procedures.

Often more than one type of abuse is experienced, for example a child may be emotionally abused through threats not to tell someone about the physical or sexual abuse they are experiencing. Staff and volunteers are encouraged to be mindful of this.

Digital Safety: Abuse can be perpetrated face to face, over the phone, in writing and online. For guidance on online safety, see our [Digital Safeguarding Policy](#) and [Social Media Policy](#).

3. Who Perpetrates Abuse

Anyone may be an abuser. They may be a stranger to the child but most commonly, they are known to the child. It could be:

- A parent, step-parent or carer
- Another family member or family friend
- Neighbour
- Paid care worker
- Staff or volunteers of an organisation
- Another child such as a friend, school peer or service user

Staff Conduct: All BYO staff and volunteers undergo vetting as outlined in our [Recruitment, Selection and Onboarding Policy](#) and [Vetting and DBS Policy](#), and are expected to maintain standards outlined in our [Code of Conduct Policy](#).

4. Recognising Signs of Abuse

Children who are being abused may not understand what they are experiencing is abuse. They may also be too afraid to tell someone or not feel that there is anyone they can trust. They may feel shame or confusion and be wary of what people think. It is therefore important that as adults, we notice things that may be a sign of abuse. Below are some common signs. Depending on the child and how well you know them, there may be other signs you will notice. Be vigilant and always talk to the DSL or DDSL if you have concerns.

Common Signs Include:

- Flinching in response to sudden but harmless actions, for example someone raising a hand quickly
- Showing fear of particular places or making excuses to avoid particular people
- Knowledge of 'adult issues' for example alcohol, drugs and/or sexual behaviour which is inappropriate for their age or stage of development
- Angry outbursts or behaving aggressively towards other children, adults, animals or toys

- Becoming withdrawn or appearing anxious, clingy or depressed
- Self-harming or thoughts about suicide
- Changes in eating habits or developing eating disorders
- Regularly experiencing nightmares or sleep problems
- Regularly wetting the bed or soiling their clothes
- Behaviour that may place them at risk such as substance misuse or criminal activity
- Running away or regularly going missing from home or care
- Not receiving adequate medical attention after injuries

Bullying Note: Some signs may indicate bullying - refer to our **Anti-Bullying Policy** for specific guidance on recognising and responding to bullying.

5. Responding to Safeguarding Concerns

When a child or young person wants to tell you about what is happening to them, it is important that you create a safe, open and listening space where they know they have your attention and that you are going to take them seriously. Below are some good pointers to follow:

Do:

- Stay calm and listen carefully
- Reassure them in words and in your body language
- Tell them they have done the right thing telling you
- Encourage them to talk and do not interrupt whilst they are recalling events
- Ask questions only to clarify your understanding of what you are being told. This is to make sure you understand what they are telling you. You should not try to investigate or confront the alleged abuser
- Never promise to keep the information secret. Explain that you have to pass the information on to those who can help but that it will be treated confidentially (i.e. on a need to know basis)
- Explain to the child what you will do next
- As soon as you can, write down the key points, using the child's words where possible
- Contact the DSL or DDSL and share the information with them as soon as possible. It is your responsibility to do so in order for BYO to carry out its responsibilities to keep children safe

Helpful Things to Say:

- "You are doing the right thing to tell me"
- "It's not your fault"
- "I'm concerned about you. It's important you are safe"
- "Thank you for telling me about this. It must have been difficult but I'm really proud of you"

Do NOT Say:

- "Why didn't you tell anyone before?"
- "I can't believe it!"
- "Are you sure about this?"
- "You could get someone into a lot of trouble"

(See Flowchart at Appendix 2)

Supporting Ongoing Disclosure

Sometimes children may tell us pieces of information at different times, they may not disclose everything. Make sure that you always reassure them and follow the above points. Share the information with the DSL or DDSL and never push the child to tell you more.

If a child talks to you and you suspect they may be about to tell you about abuse but they do not say anymore, let them know that you or other staff or volunteers at BYO are always ready to listen to them. Tell them about how they can contact Childline on 0800 1111 at any time if they are worried about something.

Creating Safe Environments

Do your best to remove any barriers to the child telling you. Follow the BYO **Code of Conduct Policy** so that you are a role model, approachable and treat the children with respect at all times. Some children may need additional support with speech or language such as being mindful if they prefer to sign or making sure there is someone who understands their mother tongue.

Inclusion Note: For guidance on supporting children with additional needs, see our **Equality, Diversity and Inclusion Policy** and **Organisational Values and Inclusion Framework**.

What You Must NEVER Do:

Keeping children safe is BYO's priority. To help us do that, you should never:

- Trivialise or dismiss your concerns
- Normalise it
- Think it will sort itself out or go away

6. Managing Allegations About Staff or Volunteers

If an allegation is made against a member of staff or volunteer it should **immediately be reported** to the DSL.

The DSL should contact the Local Authority Designated Officer (LADO) within **1 working day of an incident arising** where an allegation or concern appears that a person who works with children, has:

- Behaved in a way that has harmed a child or may have harmed a child
- Possibly committed a criminal offence against or related to a child
- Behaved in a way that indicates they may pose a risk to children
- Behaved or may have behaved in a way that indicates they may not be suitable to work with children

The LADO Administration Team can be contacted on **0121 675 1669** email: lado.secure@birmingham.gcsx.gov.uk.

See Also: Full procedures are detailed in our **Managing Allegations about Staff and Volunteers Policy**.

7. Recording Guidance

Whenever concerns are raised about a child or young person, however they arise, it is crucial to make and keep an accurate record using the appropriate documentation:

When to Use the Safeguarding Case Report Form

The BYO Safeguarding Case Report Form must be used for all:

- **Formal safeguarding concerns** requiring DSL involvement and potential external referral
- **Allegations of abuse** (physical, sexual, emotional, neglect)
- **Serious incidents** involving harm or risk of harm to children
- **Disclosures** of abuse made by children or young people
- **Concerns about child exploitation** or trafficking
- **Incidents requiring** police or local authority involvement
- **Safeguarding allegations** against staff or volunteers involving children
- **Peer-to-peer abuse** incidents requiring formal investigation
- **Any incident** where external agencies may need to be contacted

When to Use Simple Incident Logs

Simple incident logs (as outlined in our Standing Operating Procedure) are appropriate for:

- Minor accidents or injuries requiring first aid only
- Minor behavioural incidents managed through normal procedures
- Equipment issues or facility problems
- General operational incidents without safeguarding implications

Important: If you are unsure which form to use, **always err on the side of caution and use the Safeguarding Case Report Form**. The DSL can determine if the level of documentation was appropriate.

Completing the Safeguarding Case Report Form

The form must be completed:

- **As soon as possible** after the incident or disclosure (within 24 hours maximum)
- **By the person** who witnessed the incident or received the disclosure
- **Using the child's own words** where a disclosure has been made
- **With all sections** completed as fully as possible
- **In clear, factual language** avoiding opinions or assumptions

Essential information to include:

- **Date, time, and location** of incident or disclosure
- **Who was present** during the incident or disclosure
- **Exact details** of what happened or what was said
- **Observable facts** including injuries, behaviour, or demeanour
- **Actions taken** immediately following the incident
- **Who has been informed** and when

Form Distribution and Storage**Once completed, the Safeguarding Case Report Form must:**

1. **Be submitted to the DSL** immediately (within 24 hours of completion)
2. **Original copy** given to DSL for secure safeguarding filing
3. **No copies retained** by the person completing the form
4. **Electronic copies** deleted securely after DSL confirmation of receipt
5. **Stored in accordance** with Data Protection Policy requirements

The DSL will:

- **Review the form** and determine appropriate action
- **Store securely** in confidential safeguarding files
- **Share appropriately** with external agencies if required
- **Maintain confidentiality** in accordance with data protection requirements
- **Use the information** to inform risk assessments and safety planning

Training on Form Use**All staff and volunteers will receive training on:**

- **When to use** the Safeguarding Case Report Form vs. simple incident logs
- **How to complete** the form accurately and comprehensively
- **Submission procedures** and time-scales
- **Confidentiality requirements** and secure handling
- **Follow-up procedures** after form submission

Regular refresher training will be provided to ensure consistent and appropriate use of safeguarding documentation.

DSL Section

9. Logging concerns and action

When we have concerns that a child, young person or adult may be at risk of harm or not safe or abused these concerns must be reported without delay to BYO DSL/DDSL.

All safeguarding concerns must be recorded and held securely and confidentially.

Once the staff member or volunteer has shared their concerns with the DSL/DDSL a record of the details of the referral must be logged by the DSL/DDSL.

The DSL will review the information received, including any other information already known about the child in order to decide the appropriate course of action. Where unsure, the DSL will contact Birmingham Children's Advice Services for advice. It is possible to seek advice from Children's Services without disclosing details of the child concerned. The DSL can also contact NSPCC on 0800 800 5000 for advice.

If the decision at this time is not to refer to the local authority a record should be made as to why this was the decision and who agreed this. This must be agreed by a designated senior person or Director.

10. Making a Referral to Children's Social Care

It is the responsibility of the DSL to make a referral to Children's Services where they have concerns about a child who may be experiencing or at risk of abuse.

All referrals must be supported by:

- **Completed Safeguarding Case Report Form** providing comprehensive incident details
- **Additional supporting documentation** where available (witness statements, photographs of injuries if appropriate)
- **Previous safeguarding case report forms** relating to the same child where relevant
- **Risk assessment information** based on documented concerns

The Safeguarding Case Report Form provides:

- **Structured information** in the format required by Children's Services
- **Comprehensive evidence base** for referral decisions
- **Professional documentation** suitable for potential legal proceedings
- **Clear timeline** of events and actions taken
- **Contact details** and consent information required for investigation

Where the incident/disclosure occurs out of hours and the child or young person is at immediate risk:

1. **Contact emergency services (999)** if immediate danger
2. **Contact Children's Services** out of hours team
3. **Complete Safeguarding Case Report Form** as soon as safely possible
4. **Submit to DSL** immediately upon their availability
5. **Follow up** with formal referral using case report form information

Birmingham Children's Services Contact:

- **Multi-Agency Safeguarding Hub (MASH):** 0121 303 1888
- **Out of Hours:** 0121 675 4806
- **Online referral:** Available through Birmingham City Council website

Referral Follow-up Using Case Report Forms:

Once a referral has been made, the DSL will:

- **Track referral progress** using case reference numbers
- **Provide additional information** from case report forms as requested
- **Update case report forms** with external agency actions and outcomes
- **Maintain comprehensive records** of all referral communications
- **Use case report data** to monitor effectiveness of referral processes

11. Form Management

Safeguarding Case Report Form Management

The DSL is responsible for:

Form Receipt and Review

- Receiving all completed Safeguarding Case Report Forms within 24 hours of incident
- Reviewing forms for completeness and clarity
- Following up with form submitters if additional information needed
- Determining appropriate action based on form content and risk assessment

Form Processing

- Logging all forms in the secure safeguarding register
- Assigning case reference numbers for tracking and monitoring
- Initiating external referrals where required based on form content
- Coordinating investigations using form information as evidence base
- Updating risk assessments based on incidents recorded

Information Sharing

- Sharing form information with relevant external agencies (police, local authority, LADO)
- Providing copies to investigators while maintaining confidentiality
- Redacting information where necessary to protect third parties
- Maintaining audit trail of all form sharing and distribution

Storage and Retention

- Secure storage in locked filing systems with restricted access
- Electronic copies stored on password-protected systems only
- Retention in accordance with safeguarding record retention schedules
- Secure destruction when retention period expires

Quality Assurance

- Regular review of form completion quality and consistency
- Feedback to staff on form completion where improvements needed
- Training updates based on form analysis and learning
- Annual audit of safeguarding case report form processes

12. Staff Training Requirements

Safeguarding Case Report Form Training

All staff and volunteers must receive training on:

Form Recognition and Usage

- **Identifying situations** requiring Safeguarding Case Report Form completion
- **Distinguishing between** safeguarding incidents and general operational incidents
- **Understanding the legal importance** of accurate safeguarding documentation
- **Recognising time-sensitive** nature of form completion

Form Completion Skills

- **Accurate recording** of facts without opinion or assumption
- **Using appropriate language** including child's own words in disclosures
- **Completing all sections** thoroughly and legibly
- **Understanding confidentiality** requirements during completion

Submission and Follow-up

- **Secure submission procedures** to DSL within required time-scales
- **Understanding next steps** after form submission
- **Confidentiality maintenance** during and after form completion
- **Recognition that form completion** does not end safeguarding responsibilities

Ongoing Competence

- **Annual refresher training** on form use and safeguarding documentation
- **Scenario-based practice** in completing forms for different incident types
- **Quality feedback** on completed forms to improve accuracy
- **Updates on legal requirements** and best practice in safeguarding recording

Training records will be maintained showing all staff and volunteer completion of safeguarding case report form training and regular refresher sessions.

Radicalisation Concerns

For concerns regarding radicalisation, the DSL will contact the local Prevent team for support and advice: https://www.birmingham.gov.uk/the_prevent_duty

Appendix 1: Signs and Types of Child Abuse

1. Physical abuse

Definition

Physical abuse occurs when a child is deliberately harmed, resulting in injuries such as bruises, cuts, burns, or broken bones. It may involve hitting, kicking, shaking, throwing, poisoning, burning, or suffocating. It also includes fabricated or induced illness (FII), where a parent or carer deliberately causes or pretends a child is unwell.

Signs that may indicate physical abuse:



- Bruises on babies not yet crawling or walking
- Bruising on cheeks, ears, palms, arms, or feet
- Bruises on back, buttocks, hips, thighs, or backs of legs
- Multiple bruises in clusters or patterned shapes (e.g. finger marks)
- Large oval-shaped bite marks
- Burns with clear shapes of objects (e.g. cigarette burns)
- Burns to backs of hands, feet, genitals, or buttocks
- Injuries at different stages of healing
- Injuries inconsistent with explanation given
- Delay in seeking medical help for visible injuries

Safeguarding Note

Physical abuse may be hidden, minimised, or excused by the child or adult. Patterns, timing, or injury location are often key clues. Use professional curiosity and look beyond surface explanations.

Action Prompt

If physical abuse is suspected, record your observations using the Safeguarding Case Report Form and alert the DSL or DDSL immediately. Do not attempt to investigate or challenge the suspected abuser yourself.

2. Domestic violence or abuse

Domestic violence is abusive, controlling, bullying, or violent behaviour between people in intimate or familial relationships. **When children are exposed to domestic violence, it is a form of child abuse.**

Children who witness domestic abuse may:



- Become aggressive
- Display anti-social behaviour
- Suffer depression or anxiety
- Not do well at school
- Experience disruption from moving to refuges

Domestic violence includes repeated patterns of controlling, coercive, or threatening behaviour, often between partners or family members, regardless of gender or sexuality. It can include forms such as honour-based abuse, female genital mutilation (FGM), and forced marriage.

Coercive behaviour can include:

- Acts of assault, threats, humiliation and intimidation
- Harming, punishing, or frightening the person
- Isolating the person from sources of support
- Exploitation of resources or money
- Preventing the person from escaping abuse
- Regulating everyday behaviour

Possible indicators of domestic abuse in children:

- Low self-esteem
- Belief that abuse is their fault
- Unexplained injuries or damage to property
- Emotional outbursts or persistent fear
- Avoiding friends or social situations
- Overhearing or witnessing abusive behaviour
- Verbal abuse in front of the child
- Limited access to food, clothing, or finances

Safeguarding Note

Coercion and control are often **hidden and gradual**. Look at the **broader context** of a child's home life and relationships – not just isolated events. Children may normalise what they experience.

Action Prompt

If you suspect domestic abuse, act immediately. Follow BYO safeguarding procedures and record concerns in the **Safeguarding Case Report Form**. Escalate to the **DSL or DDSL** for appropriate support and intervention.

3. Child Sexual Abuse

Definition

Sexual abuse involves forcing, coercing, or manipulating a child into taking part in sexual activities. It does not always involve physical contact and may happen in person or online. The child may not understand what is happening or that it is abusive.

Contact abuse includes:



- Sexual touching of any part of the body
- Rape or attempted penetration
- Forcing or coercing sexual acts
- Making a child touch someone else's genitals

Non-contact abuse includes:

- Encouraging or forcing a child to watch or listen to sexual activity
- Showing pornography to a child
- Making, viewing, or sharing child sexual abuse images
- Online sexual abuse and grooming

Physical signs may include:

- Anal or genital soreness, itching, or bleeding
- Bruising in intimate areas
- Pain or discomfort when walking or sitting
- Unusual discharge or infections
- Sexually transmitted infections (STIs)
- Pregnancy

Behavioural signs may include:

- Inappropriate sexual knowledge for their age
- Sexualised behaviour or language
- Becoming withdrawn, anxious, or aggressive
- Avoiding specific individuals or places
- Sudden changes in mood or school performance

Safeguarding Note

Sexual abuse is rarely disclosed directly. Children may feel ashamed, confused, or fearful. Look for patterns and indicators — a trauma-informed, consent-sensitive approach is essential. Treat all suspicions seriously.

Action Prompt

If sexual abuse is suspected or disclosed, **act immediately**. Follow BYO safeguarding procedures and document all concerns using the **Safeguarding Case Report Form**. Report directly to the **DSL or DDSL** for appropriate referral and support.

4. Psychological or Emotional Abuse

Definition

Emotional abuse is a persistent pattern of behaviour that severely impairs a child's emotional development, sense of self-worth, and overall mental well-being. It may not leave visible signs but can cause deep and lasting harm.

Types of Psychological Abuse



- Humiliating, putting down, or constantly criticising
- Shouting, threatening, or name-calling
- Mocking or forcing a child to perform degrading tasks
- Scapegoating or blaming the child unfairly
- Attempting to control a child's thoughts, choices, or behaviours
- Preventing friendships or social interaction
- Regularly exposing a child to distressing events or hostility
- Being emotionally unavailable or cold

Possible Indicators of Emotional Abuse

- Being overly affectionate or clingy with strangers
- Difficulty forming secure attachments with caregivers
- Low self-esteem, anxious behaviour, or fearfulness
- Struggling to play or interact socially with other children
- Sudden outbursts or aggressive behaviour
- Over-sensitivity to correction or feedback
- Fear of making mistakes
- Signs of self-harm or emotional withdrawal

Safeguarding Note

Emotional abuse often takes place over time and may not be immediately obvious. It may coexist with other types of abuse, particularly neglect or physical abuse. Patterns of behaviour are key – trust your instincts and look for cumulative concerns.

Action Prompt

If you suspect emotional abuse, record your observations clearly using the **Safeguarding Case Report Form**. Do not delay reporting – escalate your concerns to the **DSL or DDSL** for assessment and support planning.

5. Female Genital Mutilation (FGM)

Definition

FGM refers to the partial or total removal of external female genitalia for non-medical reasons. It is considered a form of child abuse and is illegal in the UK. FGM is a severe violation of human rights and has long-term consequences for health and well-being.

A child at immediate risk may talk about:

- A long trip abroad or visiting extended family
- A special ceremony to “become a woman”
- A female relative being cut
- A visitor (e.g. elder or ‘cutter’) coming from abroad
- Missing school repeatedly for unexplained reasons

A girl who has had FGM may:

- Have difficulty walking, sitting, or standing
- Spend longer than usual in the bathroom/toilet
- Show signs of emotional distress: withdrawn, anxious, or low mood
- Avoid physical activity or PE
- Display reluctance to undergo medical examinations
- Show a change in behaviour following absence

Legal Requirement: All professionals (including teachers, social workers, and health staff) have a legal duty to report known cases of FGM in girls under 18 to the police, under the Serious Crime Act 2015.

Safeguarding Note

FGM is often carried out in secret and may involve strong cultural or familial pressure. It may be accompanied by shame, fear, or misinformation. Children may not fully understand what has happened or may be afraid to speak. A calm, supportive, and non-judgemental response is essential.

Action Prompt

If you suspect that a child is at risk of FGM, or has already undergone FGM, do not delay. Record your concerns using the **Safeguarding Case Report Form** and report directly to the **DSL or DDSL**. Known cases must also be reported to the **police** without delay.

6. Discriminatory Abuse

Definition

Discriminatory abuse involves harmful or unfair treatment based on a person's protected characteristics, such as age, disability, race, religion or belief, gender identity, or sexual orientation. It can take the form of harassment, slurs, exclusion, or unequal access to services, and is a safeguarding issue under the Equality Act 2010.

Types of Discriminatory Abuse



- Unequal treatment based on protected characteristics
- Use of derogatory, demeaning, or offensive language
- Denial of access to communication aids or interpretation services
- Harassment, name-calling, or deliberate exclusion
- Denial of access to education, healthcare, or justice
- Poor or substandard care based on cultural, racial, or identity-based prejudice

Possible Indicators of Discriminatory Abuse

- Avoidance of services or certain environments
- Distress or withdrawal when specific topics or people are mentioned
- Expressions of fear, anger, or frustration linked to identity
- Unexplained decline in mood or well-being
- Reported use of slurs or discriminatory language
- Isolation from peers or staff
- Lack of cultural, religious, or identity-based support needs being met
- Services or care failing to reflect personal values or protected characteristics

Safeguarding Note

Discriminatory abuse can be both individual and systemic. It may be overt or subtle, but its impact on dignity, trust, and safety is significant. This form of abuse is often under-reported due to fear of not being believed or of retaliation. A proactive safeguarding response is essential.

Action Prompt

If you witness or suspect discriminatory abuse, report it immediately to the DSL or DDSL. Clearly record what you have seen or heard using the Safeguarding Case Report Form. All incidents must be treated as safeguarding concerns and addressed with urgency, dignity, and respect.

7. Child Sexual Exploitation (CSE)

Child Sexual Exploitation (CSE) is a form of sexual abuse where children are manipulated or coerced into sexual activity in exchange for things like attention, gifts, money, alcohol, or affection. Exploitation can take place online or in person and often involves grooming and control, where the child may not recognise the abuse.

Signs may include:

- Going missing from home, care, or school
- Unexplained possessions, money, or clothing
- Older partners or friendships with risky adults
- Frequenting places of concern (e.g. known exploitative environments)
- Evidence of drug or alcohol misuse
- Involvement in petty crime or gang association
- Disclosure of abuse or sexually inappropriate relationships
- Unexplained injuries or changes in emotional state

8. Harmful Sexual Behaviour (HSB)

Harmful Sexual Behaviour (HSB) refers to sexual actions by children or young people that are developmentally inappropriate, coercive, abusive, or violent. This includes peer-on-peer abuse, and may harm both the child displaying the behaviour and others around them.

HSB can include:

- Use of sexually explicit language or jokes that are aggressive or persistent
- Inappropriate sexual touching or exposure
- Sexual violence, threats, or coercive behaviours
- Seeking sexual contact with much younger children or adults
- Behaviour that is persistent, secretive, or involves manipulation

Note: Sexual behaviour is considered harmful if it involves force, coercion, secrecy, or a power imbalance (including a significant age gap of 2+ years).

Safeguarding Note

CSE and HSB both carry serious risk and emotional trauma. Victims of CSE may not see themselves as abused, while children displaying HSB may be imitating abuse they've experienced. Responses must be child-centred, trauma-informed, and never dismissive or punitive. Multi-agency working is essential for safety and support.

Action Prompt

If you suspect CSE or HSB, report the concern immediately to the DSL or DDSL. Record what has been said, observed, or disclosed using the Safeguarding Case Report Form. Ensure supervision and safety for all involved, and avoid confrontation. The DSL may refer to children's social care, the police, and where appropriate, CAMHS or sexual trauma services.

9. Neglect and Acts of Omission

Definition

Neglect is the ongoing failure to meet a child's basic physical or emotional needs. It can occur at any age and may severely affect a child's health, development, or emotional well-being. Neglect includes failing to provide adequate food, clothing, shelter, supervision, medical care, or emotional warmth.

Types of Neglect and Acts of Omission



- Failing to provide nutritious food or appropriate clothing
- Leaving the child unsupervised or exposed to dangerous situations
- Not seeking medical attention when a child is ill or injured
- Ignoring emotional needs, comfort, or distress
- Failing to support the child's education or attendance
- Not ensuring the child has clean, safe living conditions
- Withholding affection or stimulation vital to development

Possible Indicators of Neglect in Children

- Constant hunger, tiredness, or complaints of being cold
- Poor hygiene, dirty clothing, or untreated medical issues
- Frequent absences or lateness to school
- Developmental delays or learning difficulties
- Being withdrawn, anxious, or showing low self-esteem
- Inappropriate clothing for weather conditions
- Taking on caring responsibilities inappropriate for their age
- Clinginess or indiscriminate affection toward unfamiliar adults

Safeguarding Note

Neglect can be difficult to detect because it often happens over time. Consistent observation, curiosity, and professional challenge are essential. Children may not always realise they are being neglected.

Action Prompt

If you have concerns that a child is being neglected, report it immediately to the Designated Safeguarding Lead (DSL). Use the Safeguarding Concern Form and record what you have seen, heard, or observed in clear, factual language.

10. Self-Neglect

Definition

While self-neglect is more common in adults, in children and young people — particularly older adolescents — it may present as a persistent inability or refusal to care for personal health, hygiene, or safety. It can be a sign of deeper emotional distress, trauma, or unmet care needs.

Types of Self-Neglect in Young People



- Persistent refusal to wash or change clothing
- Refusing food or avoiding eating, leading to weight loss or malnutrition
- Living in unsafe or unsanitary environments (e.g. hoarding, squalor)
- Avoiding medical care despite illness or injury
- Withdrawal from all support networks or services
- Repeated self-harming behaviour without seeking help
- Failing to manage daily tasks or personal responsibilities in older adolescents

Possible Indicators of Self-Neglect

- Extremely poor hygiene or strong body odour
- Clothing that is consistently dirty, damaged, or inappropriate for the weather
- Signs of hunger, malnutrition, or dehydration
- Health needs left untreated (e.g. infections, injuries, or chronic illness)
- Avoiding school or health appointments
- Signs of hoarding or animal neglect in the home
- Repeated expressions of hopelessness, shame, or worthlessness
- Refusing all support from professionals or carers

Safeguarding Note

In children, what appears as “self-neglect” is often a response to wider safeguarding risks such as neglect, abuse, trauma, or mental health needs. Always explore the underlying causes, not just the presenting behaviour.

Action Prompt

If you suspect a child is displaying signs of self-neglect, refer the concern to the Designated Safeguarding Lead (DSL). A multi-agency approach may be necessary, and support should focus on understanding the child’s emotional and environmental context.

11. Child Trafficking and Modern Slavery

Definition

Child trafficking involves the recruitment, transportation, transfer, harbouring, or receipt of a child for the purpose of exploitation. Children can be trafficked across borders or within the UK, often without recognising they are victims. Trafficking is a criminal offence and a serious safeguarding concern.

Children are trafficked for:



- Sexual exploitation
- Benefit fraud
- Forced marriage
- Domestic servitude (e.g. cleaning, childcare, cooking)
- Forced labour (e.g. agriculture, car washes, nail bars)
- Criminal exploitation (e.g. drug running, theft, begging)

Possible Indicators of Child Trafficking / Modern Slavery

- Rarely allowed out alone or not attending school
- Appears to live or sleep in the place of work
- Unaware of address or uncertain about where they are
- Has no access to parents or trusted adults
- Under control of another person when speaking to professionals
- Evidence of poor physical health or untreated medical issues
- In possession of false documents or none at all
- Lives in overcrowded or unsuitable accommodation
- Signs of emotional trauma, fear, or distrust of adults
- Works long hours, often for little or no pay
- Seen in locations known for exploitation or criminal activity

Safeguarding Note

Children who are trafficked may not disclose their situation. They may appear compliant, distrustful, or confused. Prioritise building trust, maintaining safety, and working closely with partner agencies.

Action Prompt

If you suspect a child may be a victim of trafficking or modern slavery, follow BYO safeguarding procedures and refer immediately to the DSL or DDSL. Record concerns clearly using the Safeguarding Case Report Form. A multi-agency response may include police, the local authority, and the Home Office via the National Referral Mechanism (NRM).

12. Hate Crime

Definition

Hate crime is any criminal offence that is perceived by the child or others to be motivated by prejudice or hostility towards their race, religion, disability, gender identity, or sexual orientation. For children, this may occur in schools, neighbourhoods, public spaces, or online. It can be emotional, verbal, physical, or psychological – and must always be taken seriously as a safeguarding concern.

Types of Hate Crime Abuse



- Verbal bullying, insults, or racist/homophobic language
- Physical assault or intimidation
- Threats or gestures based on identity
- Online abuse, memes, or targeted harassment
- Social exclusion or peer-group targeting
- Vandalism or graffiti related to the child's background or beliefs

Possible Indicators of Hate Crime

- Withdrawal from peers or reluctance to attend school or activities
- Anxiety, fear, or changes in mood without clear reason
- Becoming tearful or distressed when certain topics are discussed
- Disclosures about being bullied or called names due to their background
- Signs of injuries or belongings being damaged or stolen
- Unexplained drop in confidence, performance, or attendance

Safeguarding Note

Hate incidents involving children may be dismissed by peers or adults as “just teasing,” but persistent targeting based on identity is abusive. Early intervention is vital to protect the child's well-being and to challenge discrimination.

Action Prompt

If a hate-related incident is disclosed or suspected, report it to the Designated Safeguarding Lead (DSL) without delay. Record what the child says in their own words and monitor for escalation. The DSL will consider appropriate support, family engagement, and whether external referrals (e.g. police, social care, PREVENT) are required.

13. Radicalisation

Definition

Radicalisation is the process by which a child or young person is influenced to adopt extreme ideologies, potentially leading them to support violence, terrorism, or hatred toward others. It may happen online or in person and often involves grooming, manipulation, or exposure to harmful content. Schools and youth organisations have a duty to prevent this under the Prevent strategy.

Types of Radicalisation Risk in Children



- Being targeted online by extremist individuals or groups
- Viewing or sharing extremist materials, memes, or videos
- Feeling isolated or alienated from peers or wider society
- Expressing admiration for groups promoting hate or violence
- Dramatic changes in appearance, language, or behaviour
- Using language that shows intolerance or justification of violence

Possible Indicators of Radicalisation

- Becoming withdrawn or unusually quiet
- Talking as if repeating scripted or rehearsed opinions
- Showing signs of anger or intolerance toward specific groups
- Sudden disrespect for authority or rules
- Obsession with conspiracy theories or world events
- Spending large amounts of time on closed forums or secretive online activity
- Expressing feelings of persecution or injustice

Safeguarding Note

Radicalisation in children is often subtle and may stem from feelings of loneliness, bullying, or frustration. It is not a child protection issue because of their beliefs – but because they may be vulnerable to manipulation and exploitation.

Action Prompt

If you are concerned that a child may be at risk of radicalisation, report it to the Designated Safeguarding Lead (DSL) immediately. Do not confront the child – instead, record your observations and allow the DSL to assess risk and, where needed, make a referral under the Prevent duty.

14. Fabricated or Induced Illness

Definition

Fabricated or Induced Illness (FII) is a form of child abuse where a parent or carer deliberately exaggerates, fabricates, or causes symptoms of illness in a child. This is done to gain attention, control, or access to medical treatment. FII can cause serious physical and emotional harm and may go undetected without careful observation and professional curiosity.

Types of FII



- Claiming the child has symptoms that are not observed by others
- Withholding food, medication, or sleep to induce illness
- Giving the child substances to make them ill (e.g. laxatives, sedatives)
- Interfering with medical care (e.g. test results, medication doses)
- Repeatedly seeking unnecessary treatments or procedures
- Moving from professional to professional, dismissing previous advice

Possible Indicators of FII

- Frequent unexplained illnesses or hospital visits
- Discrepancies between medical findings and reported symptoms
- Symptoms only appearing in the presence of the parent or carer
- Carer appears unusually knowledgeable, controlling, or insistent
- Child shows anxiety about medical procedures or being examined
- Carer resists professionals speaking to the child alone

Safeguarding Note

FII can be complex and difficult to recognise. Concerns may emerge gradually, and professionals should trust their instincts, record concerns diligently, and discuss patterns with the Designated Safeguarding Lead.

Action Prompt

If FII is suspected, speak with the DSL immediately. Do not confront the parent. Maintain accurate, factual records, and refer for multi-agency involvement. Health services and social care must work together to assess risk and protect the child.

15. Bullying and Cyberbullying

Definition

Bullying is repeated, intentional behaviour that hurts, intimidates, or humiliates a child. It often involves a real or perceived power imbalance. Bullying can happen in person or online (cyberbullying), and both forms can have serious emotional and mental health impacts.

Types of Bullying (including Cyberbullying)



- **Verbal:** Name-calling, taunting, mocking, or threats
- **Physical:** Hitting, pushing, spitting, or damaging property
- **Emotional:** Excluding, isolating, spreading rumours, or manipulation
- **Non-verbal:** Staring, glaring, gestures meant to intimidate
- **Cyberbullying:** Use of phones, messaging apps, or social media to harm

Examples of Cyberbullying:

- Sending abusive or threatening texts or DMs
- Sharing embarrassing photos or videos
- Posting false or harmful content online
- Creating fake profiles to impersonate or target a child
- Excluding someone from group chats or online activities
- Encouraging harmful behaviours, such as self-harm
- Trolling or harassment on gaming or social platforms

Possible Indicators of Bullying or Cyberbullying

- Becoming withdrawn, anxious, or unusually quiet
- Avoiding school, clubs, or online activity
- Sudden drop in confidence or academic performance
- Changes in eating or sleeping patterns
- Visible injuries or damage to belongings
- Secrecy around phone or internet use

Safeguarding Note

Bullying can be dismissed as “just banter” or “falling out,” but persistent or targeted behaviour must always be taken seriously. Online abuse may be hidden, escalate quickly, and coexist with other safeguarding concerns.

Action Prompt

If you are concerned that a child is being bullied or cyberbullied, report it to the DSL or DDSL immediately. Record what you have seen, heard, or been told. Where online content is involved, preserve screenshots or messages where possible, and refer to the Anti-Bullying Policy for additional guidance.

See Also: Our [Anti-Bullying Policy](#) provides detailed guidance on preventing and responding to bullying.

16. County Lines / Criminal Exploitation

Definition

County Lines is a form of criminal exploitation where children are groomed and manipulated by gangs to carry drugs, money, or weapons from urban areas to suburban or rural locations. This often involves coercion, threats, violence, and emotional manipulation. Children may be moved away from home and placed in dangerous environments.

Types of Exploitation



- Being sent to different areas to distribute drugs ("running")
- Staying in unsafe or unknown accommodation (hotels, Airbnbs, trap houses)
- Involvement in weapons or drug production
- Using rail, bus, or taxi routes for gang-related movement
- Having their home or a vulnerable adult's home taken over for drug activity ('cuckooing')

Possible Indicators of Criminal Exploitation

- Going missing for periods of time or being found far from home
- Sudden possession of expensive items or unexplained money
- Associating with older peers or adults without clear reason
- Carrying multiple phones or using burner phones
- Truancy, disengagement from education, or drop in performance
- High levels of secrecy, anxiety, or fearfulness
- Injuries that are unexplained or inconsistent with the story
- Being picked up or dropped off at unusual hours

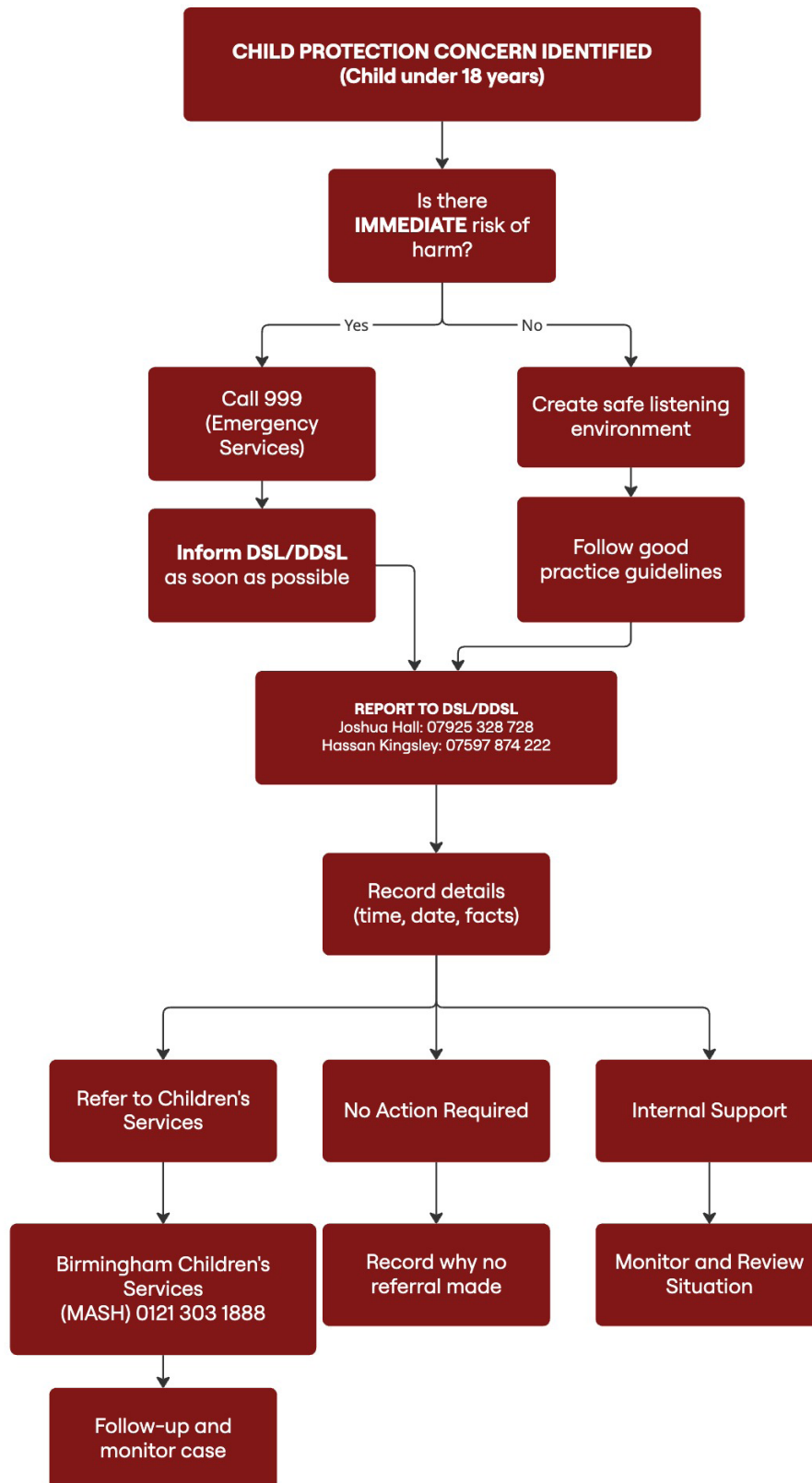
Safeguarding Note

Children caught up in County Lines may not see themselves as victims. They may appear to be making choices, but are often groomed, threatened, or emotionally dependent on exploiters. A non-judgemental, safeguarding-first approach is essential.

Action Prompt

If you suspect a child is being criminally exploited, report your concern immediately to the DSL or DDSL. This is both a safeguarding and serious criminal matter. Detailed documentation and swift multi-agency referral are critical to protect the child and disrupt exploitation.

Appendix 2 - Child Protection Reporting Flowchart



SPECIAL CASES:

- Staff/Volunteer allegation → LADO (0121 675 1669)
- Peer-to-peer abuse → Managing Allegations of Peer-to-Peer Abuse Policy
- Adult concern (18+) → Adult Safeguarding Procedures
- Bullying → Anti-Bullying Policy
- Online safety → Digital Safeguarding Policy
- Radicalisation → DSL contacts Prevent team

ALTERNATIVE REPORTING:

- If unable to report through normal channels, see **WHISTLEBLOWING POLICY**

EMERGENCY CONTACTS:

Police: 999

Children's Services Out of Hours: 0121 675 4806

Childline: 0800 1111

NSPCC: 0800 800 5000

Related Policies

This policy should be read alongside:

- Safeguarding Policy Framework
- Adult Safeguarding Procedures
- Managing Allegations about Staff and Volunteers
- Managing Allegations of Peer-to-Peer Abuse
- Anti-Bullying Policy
- Digital Safeguarding Policy
- Social Media Policy
- Code of Conduct Policy
- Code of Behaviour Policy
- Recruitment, Selection and Onboarding Policy
- Vetting and DBS Policy
- Standing Operating Procedure for CYP Events
- Health and Safety Policy
- Whistleblowing Policy
- Data Protection Policy

Document Control:

- This policy should be read in conjunction with our Safeguarding Policy Framework
- For concerns about children under 18, refer to Child Protection Procedures
- For allegations about staff conduct, refer to Managing Allegations about Staff and Volunteers Policy
- If unable to report through normal channels, see Whistleblowing Policy

This policy has been approved by Hassan Kingsley, the Governor, and will be reviewed annually or following any significant incident or change in legislation.

Reviewed by:



Hassan Kingsley
Governor

Date: 28/06/27

Next Review Date: 01/07/2027

*Britannia Youth Organisation CIC is a Community Interest Company registered in England and Wales.
Company No. 12515346 | Registered Address: 36 St Joseph's Rd, Ward End, Birmingham, B8 2JU*



Adult Safeguarding Procedures

Part of the BYO Safeguarding and Protection Policy Suite

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Introduction

Safeguarding adults at risk and promoting their welfare is of paramount importance at BYO. These procedures should be read in conjunction with our **Safeguarding Policy Framework** and apply to all staff and volunteers when planning and delivering activities with adults.

BYO delivers services for young people which includes both children (under 18) and young adults (18-24). We strive to have appropriate safeguards in place for young people and adults at all times.

See Also: For guidance on planning and delivering activities, refer to our **Health and Safety Policy** and **Offsite Visits Policy**. For digital safety concerns, see our **Digital Safeguarding Policy**.

Designated Safeguarding Lead

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Deputy Designated Safeguarding Lead

To be appointed



1. Purpose & Application

These procedures are for staff and volunteers of BYO and set out what you are expected to do when you have a safeguarding concern about an adult at risk.

Different Procedures Apply For:

- **Concerns about staff or volunteer behaviour:** Report to DSL/DDSL - **Managing Allegations about Staff and Volunteers Policy** applies
- Concerns about harmful behaviour by children/young people: Report to DSL/DDSL - **Managing Allegations of Peer-to-Peer Abuse Policy** applies
- **Safeguarding concerns about children (under 18):** Follow **Child Protection Procedures**

Whistleblowing Route: If you feel unable to report concerns directly through these channels, see our **Whistleblowing Policy** for alternative reporting options

2. Definition: Adult at Risk

An adult at risk is defined by the Care Act 2014 as someone over 18 years old who:

- Has care and support needs AND
- Is experiencing, or is at risk of, abuse or neglect AND
- As a result of their care and support needs is unable to protect himself or herself against the abuse or neglect or the risk of it

All three criteria must be met for someone to be an adult at risk. Where someone meets these criteria, the adult is entitled to seek protection and/or support from the local authority who will carry out enquiries to establish the appropriate response and support.

Care and support needs is a broad term used to describe additional support packages which might take the form of financial, care, and adaptation support to enable someone to live their life. It is not necessary for the local authority to have carried out an assessment of needs for care and support or to be providing any such support at the time of experiencing abuse, for a person to meet the criteria for adult at risk.

3. Self Determination and Mental Capacity

The law regarding safeguarding adults is different to that of safeguarding children. All adults have a right to make decisions about their own lives, even if those decisions would appear unwise. This is called the right to “**self determination**”. Careful consideration must therefore be made when making decisions about safeguarding adults, particularly the sharing information outside the organisation, if the adult does not consent to do so.

Mental Capacity refers to a person’s ability to make decisions for themselves and their own life. An adult may experience impaired mental capacity due to health or circumstances. For example, they may be affected by alcohol, be unconscious, suffer a seizure or may have dementia. The Mental Capacity Act 2005 (MCA) says that mental capacity is “**time and decision specific**”. This means we should not assume someone does not have mental capacity to make a decision because of a health condition or other factor. To protect people’s right to self determination, the assumption should always be that a person is able to make their own decisions, unless there is evidence otherwise.

4. Recognising When Adults May Be at Risk of Abuse

Abuse is a violation of a person's human rights or dignity by someone else. There are many kinds of abuse, some of which are listed below:

- Physical
- Sexual
- Psychological or emotional
- Neglect or acts of omission
- Financial or material
- Discriminatory
- Hate crime
- Institutional abuse
- Radicalisation
- Domestic Violence
- Modern Slavery
- Self Neglect and hoarding

(See Appendix 1 for detailed descriptions)

Abuse can be either deliberate or be the result of ignorance or lack of training, knowledge or understanding. It is likely that if someone is experiencing one form of abuse, they will also be experiencing another. For example, an adult being financially abused may also experience emotional abuse if they feel threatened and/or they may be physically hurt by the perpetrator.

Abuse may be perpetrated face to face, in writing, on the phone or online such as through social media or email.

Digital Safety Note: For concerns involving online abuse or digital safety, also refer to our **Digital Safeguarding Policy** and **Data Protection Policy**.

Who Perpetrates Abuse

Anyone may be an abuser. They may be a stranger to the adult but most commonly they are known to the person they abuse. They could be:

- A parent, step-parent or carer
- Partner or ex partner
- Another family member
- Neighbour
- Friend
- Colleague
- Paid care worker
- Staff or volunteers of an organisation

5. Historical Abuse

Historical abuse is sometimes known as non-recent abuse. This is where an adult makes a disclosure or allegation about abuse which occurred years ago when they were a child.

There are many reasons why children do not disclose that they are experiencing abuse. The reasons are many and complex but a common factor is coercion and fear where the child feels powerless and therefore does not tell anyone. Similarly, there may be a variety of reasons why, as an adult, the person feels able to tell.

Although the abuse occurred in the past, and in some cases many years ago, BYO will respond to allegations of historical abuse with the same rigour and speed as any other safeguarding concern. In these cases it is important to check whether the alleged perpetrator is still working with or caring for children or adults at risk due to the significant likelihood that a person who has abused in the past will continue to do so.

Allegations Against Persons in Positions of Trust

Where an allegation is made against a person in a position of trust such as:

- A foster carer
- Adoptive parent
- Residential care staff
- Teacher
- Doctor
- Police officer
- Other staff or volunteers who have or currently work with children or adults at risk

This must be reported without delay to the Local Authority Designated Officer (LADO). The LADO Administration Team can be contacted on **0121 675 1669** email: lado.secure@birmingham.gcsx.gov.uk.

6. Responding to Allegations or Suspicions of Abuse

Where a staff member or volunteer reasonably suspects or is told that an adult at risk is being, has been or is likely to be abused they must take action in line with these procedures.

IMPORTANT:

- Do not alert the alleged abuser or seek to investigate
- A formal investigation will be carried out by Adult Social Care Services and police (where a criminal offence may have been committed)
- Always establish whether there is imminent risk of harm and if so to whom
- If there is immediate and/or serious risk, you should call 999

Remember that adults have a right to self determination and may not want or be ready for others to know about what is happening.

When a Concern is Raised by the Adult Themselves

You should:

- Stay calm and listen carefully
- Reassure them in words and in your body language
- Tell them they have done the right thing telling you
- Encourage them to talk and do not interrupt whilst they are recalling events Be aware that the person's ability to recount their concern or allegation will depend on age, culture, language and communication skills and disability
- Ask questions only to clarify your understanding of what you are being told. This is to make sure you understand what they are telling you. You should not try to investigate or confront the alleged abuser
- Never promise to keep the information secret. Explain that you have to pass the information on to those who can help but that it will be treated confidentially (i.e. on a need to know basis)
- Never try to confront the alleged abuser
- In cases of suspected domestic abuse, do not advise the person to leave the relationship as this could escalate the risk. They should have support from the police and domestic violence support
- Explain why you are concerned for them
- Ask their consent to share their concerns outside of BYO (you must still tell the DSL)
- Explain what will happen next with a timescale
- As soon as you can, write down the key points, using the adult's words where possible
- Contact the DSL or DDSL and share the information with them as soon as possible. Let the DSL know whether consent has been given or not when you report the concern

Remember: It is your responsibility to report to the DSL in order for BYO to carry out its responsibilities to safeguard adults at risk.

Someone who has experienced or is experiencing abuse may not feel able to tell you initially. It is important to keep an "open door" and let the person know you or other staff and volunteers are there

to listen and support, when they are ready to tell. In the meantime, if you suspect abuse you can encourage the person to call the police if they feel they or others are in danger. In cases of suspected domestic abuse, offer to share information such as National Domestic Abuse Support Line 0800 2000 247 or other relevant organisations.

Concerns Arising From Observation

Where concerns or allegations arise other than through a disclosure (i.e. something you have seen or heard), you should contact the DSL/DDSL without delay to report your concerns.

Radicalisation Concerns

For concerns regarding radicalisation, the DSL will contact the local Prevent team for support and advice (https://www.birmingham.gov.uk/the_prevent_duty).

7. Managing Allegations About Staff or Volunteers

Where an allegation is made against a member of staff or volunteer it should **immediately be reported** to the DSL.

If an allegation is made against a member of staff or volunteer it should **immediately be reported** to the DSL who will contact the LADO for advice.

The LADO Administration Team can be contacted on **0121 675 1669** email: lado.secure@birmingham.gcsx.gov.uk.

See Also: Full procedures are detailed in our **Managing Allegations about Staff and Volunteers Policy**.

8. Recording Guidance

Where concerns are raised about an adult at risk, however they arise, it is crucial to make and keep an accurate record using the appropriate documentation:

When to Use the Safeguarding Case Report Form for Adults

The BYO Safeguarding Case Report Form must be used for all:

- Formal adult safeguarding concerns requiring DSL involvement and potential external referral
- Allegations of abuse against adults at risk (physical, sexual, emotional, financial, neglect, discriminatory)
- Serious incidents involving harm or risk of harm to adults at risk
- Disclosures of abuse made by adults at risk or on their behalf
- Concerns about adult exploitation including financial abuse or modern slavery
- Incidents requiring police or Adult Social Services involvement
- Safeguarding allegations against staff or volunteers involving adults at risk
- Domestic violence incidents involving service users
- Self-neglect concerns requiring formal intervention
- Mental capacity concerns where adults may be at risk due to impaired decision-making
- Any incident where external agencies may need to be contacted

Special Considerations for Adult Safeguarding Documentation

Mental Capacity Assessment:

- Record any concerns about the adult's mental capacity to make decisions
- Document any fluctuations in capacity or decision-making ability
- Note any decisions made in the adult's best interests
- Record consent status for information sharing and interventions

Self-Determination Considerations:

- Document the adult's wishes regarding interventions and information sharing
- Record any refusal of support or intervention
- Note any concerns about coercion or undue influence affecting decisions
- Document risk assessment where adult refuses support

Consent and Information Sharing:

- Record whether consent has been given for information sharing
- Document reasons if sharing information without consent (public interest)
- Note any family dynamics or relationships affecting safeguarding decisions
- Record any restrictions on information sharing

Completing the Form for Adult Safeguarding

Essential additional considerations for adults at risk:

- Mental capacity assessment - note any concerns about decision-making ability
- Communication needs - record any language, sensory, or cognitive communication barriers
- Support networks - document family, friends, or professional support available
- Previous concerns - note any history of abuse, neglect, or safeguarding issues
- Risk factors - identify specific vulnerabilities or circumstances increasing risk
- Adult's wishes - record the adult's preferences regarding intervention and support

Adult-Specific Information to Include

Capacity and Decision-Making:

- Assessment of mental capacity for relevant decisions
- Fluctuations in capacity or confusion
- Impact of illness, medication, or trauma on decision-making
- Evidence of coercion or undue influence

Support and Independence:

- Current living arrangements and support
- Care providers or family involvement
- Financial arrangements and management
- Social connections and isolation factors

Risk Assessment:

- Specific vulnerabilities (physical, mental, social, financial)
- Environmental factors affecting safety
- Relationship dynamics and dependencies
- Previous abuse or trauma history

DSL Section

9. Logging concerns and action

When we have concerns that a child, young person or adult may be at risk of harm or not safe or abused these concerns must be reported without delay to BYO DSL/DDSL.

All safeguarding concerns must be recorded and held securely and confidentially.

Once the staff member or volunteer has shared their concerns with the DSL/DDSL a record of the details of the referral must be logged by the DSL/DDSL.

The DSL will review the information received, including any other information already known about the adult at risk in order to decide the appropriate course of action. The DSL must establish whether the adult has given consent for the concern to be shared outside of BYO. If they have not, consideration must be given as to whether it is appropriate to go ahead and refer the matter regardless (See 'Making a Referral').

Where unsure, the DSL will contact Birmingham Adult Social Care Contact Centre for advice. It is possible to seek advice from Children's Services without disclosing details of the adult concerned. If the decision at this time is not to refer to the local authority a record should be made as to why this was the decision and who agreed this. This must be agreed by Mohammed Kaleem (Treasurer) or another designated senior person.

☎ 0121 303 1234

✉ CSAdultSocialCare@birmingham.gov.uk

You can also report concerns online through the [Birmingham Adult Social Care website](#)

10. Adult Safeguarding Case Management

Adult Safeguarding Case Report Form Management

The DSL has specific responsibilities for adult safeguarding documentation:

Adult-Specific Form Review

- Assessing mental capacity concerns recorded in forms
- Evaluating consent status and information sharing permissions
- Reviewing risk factors specific to adult vulnerabilities
- Determining appropriate interventions respecting adult autonomy

Decision-Making Documentation

- Recording decisions about information sharing without consent
- Documenting best interests decisions where capacity is impaired
- Noting consultation with relevant professionals and advocates
- Maintaining records of family or carer involvement

Multi-Agency Coordination

- Sharing forms with Adult Social Services using appropriate referral processes
- Coordinating with police where criminal matters are involved
- Liaising with healthcare professionals regarding capacity assessments
- Working with advocacy services where appropriate

Ongoing Case Management

- Monitoring adult's wishes and any changes in circumstances
- Reviewing risk assessments based on new information
- Updating care plans and support arrangements
- Tracking external agency responses and outcomes

11. Making a Referral to Adult Social Care

It is the responsibility of the DSL to make a referral to Adult Social Care Services where they have concerns about an adult at risk of abuse.

All adult safeguarding referrals must be supported by:

- **Completed Safeguarding Case Report Form** with adult-specific information
- **Capacity assessment documentation** where relevant
- **Consent records** or justification for sharing without consent
- **Risk assessment information** specific to adult vulnerabilities
- **Previous safeguarding case report forms** relating to the same adult

The Safeguarding Case Report Form provides Adult Social Care with:

- **Comprehensive background** information about the adult's circumstances
- Clear evidence of abuse, neglect, or risk factors
- **Assessment of capacity** and decision-making ability
- **Documentation of consent** status and information sharing permissions
- **Professional risk assessment** informing safeguarding planning

Where the Adult Does Not Consent

Sharing personal information without consent is a breach of confidentiality and should not be done lightly. However, data protection should never be seen as a barrier to safeguarding.

The Safeguarding Case Report Form must document:

- **The adult's specific objections** to information sharing
- **Assessment of their capacity** to make this decision
- **Risk factors** that may override their refusal
- **Professional judgment** about public interest considerations
- **Consultation with** relevant professionals or advocates

Circumstances justifying referral without consent must be clearly recorded:

a. **Risk of serious harm or death** to the individual or other person b. **Abuse or neglect of a child** c. **To assist in prevention** or detection of a serious crime d. **Legal duty** such as prevention of terrorism

Where referral is made without consent:

- **Document clear justification** in the case report form
- **Record professional consultation** and decision-making process
- **Note steps taken** to minimise information sharing to essential details
- **Maintain ongoing assessment** of the adult's capacity and wishes

Birmingham Adult Social Care Contact:

- **Adult Social Care Contact Centre:** 0121 303 1234
- **Out of Hours:** 0121 675 4806
- **Safeguarding Team:** 0121 303 1234

12. Staff Training for Adult Safeguarding Documentation

Adult-Specific Safeguarding Case Report Form Training

All staff and volunteers must receive specialised training on:

Adult Safeguarding Principles

- **Understanding self-determination** and adult autonomy in safeguarding decisions
- **Mental Capacity Act** principles and their application to safeguarding
- **Consent and information sharing** considerations specific to adults
- **Recognising adult abuse** types including financial and discriminatory abuse

Form Completion for Adult Cases

- **Documenting capacity concerns** and decision-making assessments
- **Recording adult's wishes** and preferences accurately
- **Assessing and documenting** risk factors specific to adult vulnerabilities
- **Understanding legal frameworks** affecting adult safeguarding documentation

Professional Decision-Making

- **When to override** adult's refusal of support based on capacity or risk
- **How to document** professional judgment and consultation processes
- **Understanding public interest** considerations in information sharing
- **Working with** advocacy services and family members appropriately

Multi-Agency Working

- **Understanding Adult Social Care** referral processes and requirements
- **Coordinating with** healthcare professionals on capacity assessments
- **Working with** police on potential criminal matters
- **Liaison with** legal and advocacy services

Regular case study training will include adult safeguarding scenarios requiring appropriate use of the Safeguarding Case Report Form, with emphasis on consent, capacity, and self-determination considerations.

13. Historical Abuse Documentation

Using Safeguarding Case Report Forms for Historical Adult Abuse

Historical abuse is sometimes known as non-recent abuse. This is where an adult makes a disclosure or allegation about abuse which occurred years ago when they were a child or as a vulnerable adult.

The Safeguarding Case Report Form must be used for all historical abuse disclosures and must include:

- **Current risk assessment** - whether the alleged perpetrator still poses a risk
- **Impact assessment** - how the historical abuse affects the person currently
- **Support needs** - what assistance the adult requires now
- **Criminal justice considerations** - whether police involvement is appropriate
- **Perpetrator information** - current location and access to potential victims
- **Consent for investigation** - the adult's wishes regarding formal investigation

Special considerations for historical abuse documentation:

- **Trauma-informed approach** - recognising the courage required to disclose
- **Non-judgemental recording** - avoiding questions about delay in reporting
- **Current safety assessment** - whether the person remains at risk
- **Professional support** - referral to counselling and therapeutic services
- **Legal considerations** - potential for criminal prosecution or civil proceedings

Although the abuse occurred in the past, BYO will respond to allegations of historical abuse with the same rigour and speed as any other safeguarding concern, using the Safeguarding Case Report Form to ensure comprehensive documentation for potential investigation and ongoing support planning.

Appendix 1: Signs and Types of Adult Abuse

1. Physical abuse

Definition

Physical abuse includes any deliberate act that causes physical harm or pain to an adult. This may involve force, restraint, or harmful neglect.

Types of physical abuse



- Assault, hitting, slapping, punching, kicking, hair-pulling, biting, pushing
- Rough handling
- Scalding and burning
- Physical punishments
- Inappropriate or unlawful use of restraint
- Making someone purposefully uncomfortable (e.g. opening a window and removing blankets)
- Involuntary isolation or confinement
- Misuse of medication (e.g. over-sedation)
- Forcible feeding or withholding food
- Unauthorised restraint, restricting movement (e.g. tying someone to a chair)

Possible indicators of physical abuse

- No explanation for injuries or inconsistency with the account of what happened
- Injuries are inconsistent with the person's lifestyle
- Bruising, cuts, welts, burns and/or marks on the body or loss of hair in clumps
- Frequent injuries
- Unexplained falls
- Subdued or changed behaviour in the presence of a particular person
- Signs of malnutrition
- Failure to seek medical treatment or frequent changes of GP

Safeguarding Note

Always consider patterns over time, and assess within context of the person's overall presentation, vulnerability, and capacity.

Action Prompt

If physical abuse is suspected, follow BYO procedures immediately and use the Safeguarding Case Report Form to record observations.

2. Domestic violence or abuse

Definition

Domestic violence or abuse can involve psychological, physical, sexual, financial, or emotional harm, typically within intimate or family relationships. It includes controlling, coercive or threatening behaviour, violence, or abuse between those aged 16 or over.

Key Characteristics



- Psychological
- Physical
- Sexual
- Financial
- Emotional

Domestic violence includes any incident or pattern of incidents of controlling, coercive, or threatening behaviour between intimate partners or family members, regardless of gender or sexuality. It also includes so-called 'honour'-based violence, female genital mutilation, and forced marriage. Coercive or controlling behaviour is a core part of domestic violence.

Coercive behaviour can include:

- Acts of assault, threats, humiliation and intimidation
- Harming, punishing, or frightening the person
- Isolating the person from sources of support
- Exploitation of resources or money
- Preventing the person from escaping abuse
- Regulating everyday behaviour

Possible indicators of domestic violence or abuse:

- Low self-esteem
- Feeling that the abuse is their fault when it is not
- Physical evidence of violence such as bruising, cuts, broken bones
- Verbal abuse and humiliation in front of others
- Fear of outside intervention
- Damage to home or property
- Isolation -- not seeing friends and family
- Limited access to money

Safeguarding Note

Coercive control is often subtle and cumulative. Consider the full context of a person's emotional and physical well-being when reviewing potential abuse.

Action Prompt

If domestic abuse is suspected, act immediately following BYO safeguarding procedures. Use the Safeguarding Case Report Form and inform the DSL/DDSL as required.

3. Sexual abuse

Definition

Sexual abuse is any sexual activity to which the adult has not consented, could not consent, or was pressured into consenting. It includes direct and indirect sexual acts, as well as sexual harassment or exploitation.

Types of sexual abuse



- Rape, attempted rape or sexual assault
- Inappropriate touch anywhere
- Non-consensual masturbation of either or both persons
- Non-consensual sexual penetration or attempted penetration of the vagina, anus or mouth
- Any sexual activity that the person lacks the capacity to consent to
- Inappropriate looking, sexual teasing or innuendo or sexual harassment
- Sexual photography or forced use of pornography or witnessing of sexual acts
- Indecent exposure

Possible indicators of sexual abuse

- Bruising, particularly to the thighs, buttocks and upper arms and marks on the neck
- Torn, stained or bloody underclothing
- Bleeding, pain or itching in the genital area
- Unusual difficulty in walking or sitting
- Foreign bodies in genital or rectal openings
- Infections, unexplained genital discharge, or sexually transmitted diseases
- Pregnancy in a woman who is unable to consent to sexual intercourse
- The uncharacteristic use of explicit sexual language or significant changes in sexual behaviour or attitude
- Incontinence not related to any medical diagnosis
- Self-harming
- Poor concentration, withdrawal, sleep disturbance
- Excessive fear/apprehension of, or withdrawal from, relationships
- Fear of receiving help with personal care
- Reluctance to be alone with a particular person

Safeguarding Note

Never assume someone will disclose sexual abuse openly. Look for signs and respond with sensitivity and urgency. Consent, capacity, and trauma-informed response are critical.

Action Prompt

If sexual abuse is disclosed or suspected, follow BYO safeguarding procedures immediately. Record observations using the Case Report Form and inform the DSL/DDSL at once.

4. Psychological or Emotional Abuse

Definition

Psychological or emotional abuse involves behaviour that causes emotional distress, fear, loss of dignity, or feelings of worthlessness. It can include threats, humiliation, intimidation, or isolation.

Types of Psychological Abuse



- Verbal abuse, shouting, or swearing
- Humiliation or constant criticism
- Threats of harm or abandonment
- Controlling behaviour (e.g. restricting contact with others)
- Harassment or cyberbullying
- Emotional blackmail or manipulation
- Intimidation or coercion
- Isolation from family, friends, or services

Possible Indicators of Emotional Abuse

- Low self-confidence or sudden withdrawal
- Fearfulness or anxiety, especially around certain people
- Reluctance to speak openly
- Changes in eating or sleeping habits
- Signs of depression or distress
- Unusual passivity or agitation
- Apparent false claims, by someone involved with the person, to attract unnecessary treatment
- An air of silence when a particular person is present
- Insomnia
- Low self-esteem
- Uncooperative and aggressive behaviour
- A change of appetite, weight loss/gain
- Signs of distress: tearfulness, anger

Safeguarding Note

Emotional abuse can be subtle and long-term. It often co-occurs with other forms of abuse and must be taken just as seriously.

Action Prompt

If psychological abuse is suspected, document any behavioural signs or concerns using the Safeguarding Case Report Form and escalate to the DSL or DDSL immediately.

5. Financial or Material Abuse

Definition

Financial or material abuse involves the illegal or improper use of an adult's funds, property, or assets. It can include theft, fraud, exploitation, or undue pressure in relation to wills or property.

Types of Financial Abuse



- Theft of money or possessions
- Fraud, scamming
- Preventing a person from accessing their own money, benefits or assets
- Employees taking a loan from a person using the service
- Undue pressure, duress, threat or undue influence put on the person in connection with loans, wills, property, inheritance or financial transactions
- Arranging less care than is needed to save money to maximise inheritance
- Denying assistance to manage/monitor financial affairs
- Denying assistance to access benefits
- Misuse of personal allowance in a care home
- Misuse of benefits or direct payments in a family home
- Someone moving into a person's home and living rent free without agreement or under duress
- False representation, using another person's bank account, cards or documents
- Exploitation of a person's money or assets, e.g. unauthorised use of a car
- Misuse of a power of attorney, deputy, appointee-ship or other legal authority
- Rogue trading – e.g.. unnecessary or overpriced property repairs and failure to carry out agreed repairs or poor workmanship

Possible Indicators of Financial Abuse

- Unexplained lack of money or inability to pay bills
- Sudden changes in bank account activity or withdrawal patterns
- Possessions going missing
- A carer or family member showing unusual interest in finances
- Reluctance to discuss money or financial decisions
- Lack of basic essentials (e.g. food, heating) when funds should be available
- A lack of clear financial accounts held by a care home or service
- Rent arrears and eviction notices
- Recent changes in deeds or title to property

Safeguarding Note

Financial abuse may not always involve theft – coercion, deception, and misuse of trust are just as serious and often hidden behind a façade of care.

Action Prompt

If you suspect financial abuse, do not attempt to investigate. Record concerns using the Case Report Form and refer immediately to the DSL or DDSL for appropriate action.

6. Discriminatory Abuse

Definition

Discriminatory abuse involves unfair or harmful treatment based on a person's characteristics, such as age, disability, race, religion, gender identity, or sexual orientation. It includes harassment, slurs, and exclusion.

Types of Discriminatory Abuse



- Unequal treatment based on age, disability, gender reassignment, marriage and civil partnership, pregnancy and maternity, race, religion and belief, sex or sexual orientation (known as 'protected characteristics' under the Equality Act 2010)
- Verbal abuse, derogatory remarks or inappropriate use of language related to a protected characteristic
- Denying access to communication aids, not allowing access to an interpreter, signer or lip-reader
- Harassment or deliberate exclusion on the grounds of a protected characteristic
- Denying basic rights to healthcare, education, employment and criminal justice relating to a protected characteristic
- Substandard service provision relating to a protected characteristic

Possible Indicators of Discriminatory Abuse

- Distress or withdrawal when certain topics arise
- Expressed fear of a particular person or environment
- Avoidance of services or care settings
- Unexplained deterioration in mental well-being
- Observed or reported derogatory language
- Lack of appropriate support for cultural or personal needs
- The person appears withdrawn and isolated
- Expressions of anger, frustration, fear or anxiety
- The support on offer does not take account of the person's individual needs in terms of a protected characteristic

Safeguarding Note

Discriminatory abuse may be systemic or individual. It often intersects with other abuse types and may occur in subtle or overt ways.

Action Prompt

If discriminatory abuse is witnessed or suspected, document the concern clearly and report it to the DSL/DDSL. This form of abuse is a safeguarding issue and must be addressed without delay.

7. Organisational or Institutional Abuse

Definition

Organisational abuse occurs when an institution or care setting fails to meet acceptable standards of care, resulting in harm, neglect, or loss of dignity. It can involve poor practices, neglectful routines, or authoritarian regimes within care settings.

Types of Organisational Abuse



- Discouraging visits or the involvement of relatives or friends
- Run-down or overcrowded establishment
- Authoritarian management or rigid regimes
- Lack of leadership and supervision
- Insufficient staff or high turnover resulting in poor quality care
- Abusive and disrespectful attitudes towards people using the service
- Inappropriate use of restraints
- Lack of respect for dignity and privacy
- Failure to manage residents with abusive behaviour
- Not providing adequate food and drink, or assistance with eating
- Not offering choice or promoting independence
- Misuse of medication
- Failure to provide care with dentures, spectacles or hearing aids
- Not taking account of individuals' cultural, religious or ethnic needs
- Failure to respond to abuse appropriately
- Interference with personal correspondence or communication
- Failure to respond to complaints

Possible Indicators of Institutional Abuse

- Lack of flexibility and choice for people using the service
- Inadequate staffing levels
- People being hungry or dehydrated
- Poor standards of care
- Lack of personal clothing and possessions and communal use of personal items
- Lack of adequate procedures
- Poor record-keeping and missing documents
- Absence of visitors
- Few social, recreational and educational activities
- Public discussion of personal matters
- Unnecessary exposure during bathing or using the toilet
- Absence of individual care plans
- Lack of management overview and support

Safeguarding Note

Organisational abuse may affect multiple individuals and may stem from systemic issues, not just individual actions. Whistleblowing and external oversight play a key role in uncovering these issues.

Action Prompt

If institutional abuse is suspected, escalate the concern to the DSL or DDSL and follow whistleblowing and safeguarding reporting procedures. Document everything clearly using the Safeguarding Case Report Form.

8. Neglect and Acts of Omission

Definition

Neglect is the failure to meet a person's basic needs, either deliberately or by failing to understand what they are. It includes ignoring medical, emotional, or physical care needs, withholding essentials, or failing to protect from harm.

Types of Neglect and Acts of Omission



- Failure to provide or allow access to food, shelter, clothing, heating, stimulation and activity, personal or medical care
- Providing care in a way that the person dislikes
- Failure to administer medication as prescribed
- Refusal of access to visitors
- Not taking account of individuals' cultural, religious or ethnic needs
- Not taking account of educational, social and recreational needs
- Ignoring or isolating the person
- Preventing the person from making their own decisions
- Preventing access to glasses, hearing aids, dentures, etc.
- Failure to ensure privacy and dignity

Possible Indicators of Neglect

- Poor environment – dirty or unhygienic
- Poor physical condition and/or personal hygiene
- Pressure sores or ulcers
- Malnutrition or unexplained weight loss
- Untreated injuries and medical problems
- Inconsistent or reluctant contact with medical and social care organisations
- Accumulation of undertaken medication
- Uncharacteristic failure to engage in social interaction
- Inappropriate or inadequate clothing

Safeguarding Note

Neglect may appear as a pattern rather than a single incident. It's important to assess both physical signs and the broader context of a person's care or support.

Action Prompt

If neglect or omission is suspected, document the concern promptly and escalate to the DSL or DDSL. Use the Safeguarding Case Report Form to ensure detailed recording of evidence and observations.

9. Self-Neglect

Definition

Self-neglect refers to an adult's inability or unwillingness to care for their own essential needs, which can place their health, safety, or well-being at risk. It includes hoarding, neglecting personal hygiene, or refusing necessary care or support.

Types of Self-Neglect



- Poor personal hygiene or refusal to wash
- Malnutrition or dehydration due to refusal to eat
- Unsafe living conditions (e.g. fire risk, infestations, lack of heating)
- Hoarding behaviours leading to risk
- Not taking prescribed medication or engaging with health services
- Refusing help or support despite clear risk to self
- Lack of self-care to an extent that it threatens personal health and safety
- Neglecting to care for one's personal hygiene, health or surroundings
- Inability to avoid self-harm
- Failure to seek help or access services to meet health and social care needs
- Inability or unwillingness to manage one's personal affairs

Possible Indicators of Self-Neglect

- Very poor personal hygiene
- Unkempt appearance
- Lack of essential food, clothing or shelter
- Malnutrition and/or dehydration
- Living in squalid or unsanitary conditions
- Neglecting household maintenance
- Hoarding
- Collecting a large number of animals in inappropriate conditions
- Non-compliance with health or care services
- Inability or unwillingness to take medication or treat illness or injury

Safeguarding Note

Self-neglect may result from complex causes such as mental health conditions, trauma, or capacity issues. It requires sensitive, coordinated support rather than punishment or blame.

Action Prompt

Where self-neglect is identified, record your concerns in detail and refer to the DSL or DDSL. Consider a multi-agency approach and assess mental capacity where appropriate.

10. Modern Slavery

Definition

Modern slavery is the exploitation of individuals through coercion, threats, or deception, often involving forced labour, domestic servitude, or human trafficking. It deprives people of their freedom and violates fundamental human rights.

Types of Modern Slavery



- Human trafficking (for labour, sex, or criminal activity)
- Forced labour or work under threat or coercion
- Debt bondage (working to repay an impossible or manipulated debt)
- Domestic servitude (e.g. confined and exploited in someone's home)
- Criminal exploitation (e.g. forced to steal, beg, or deal drugs)

Possible Indicators of Modern Slavery

- Signs of physical or emotional abuse
- Appearing to be malnourished, unkempt or withdrawn
- Isolation from the community, seeming under the control or influence of others
- Living in dirty, cramped or overcrowded accommodation and or living and working at the same address
- Lack of personal effects or identification documents
- Always wearing the same clothes
- Avoidance of eye contact, appearing frightened or hesitant to talk to strangers
- Fear of law enforcers

Safeguarding Note

Victims of modern slavery are often highly vulnerable and traumatised. Be sensitive, avoid confrontation, and prioritise safety and consent in any safeguarding action.

Action Prompt

If you suspect modern slavery, follow BYO safeguarding procedures immediately and refer the concern to the DSL or DDSL. Use the Safeguarding Case Report Form and seek advice from relevant external safeguarding agencies if required.

11. Hate Crime

Definition

Hate crime is any criminal offence perceived by the victim or others to be motivated by hostility or prejudice toward their race, religion, disability, sexual orientation, or gender identity. It can cause emotional, psychological, or physical harm and is both a criminal and safeguarding concern.

Types of Hate Crime Abuse



- Verbal abuse, insults, or threats
- Physical assault or intimidation
- Vandalism, graffiti, or property damage
- Threatening gestures or symbols (e.g. hate symbols or slurs)
- Abuse or harassment via social media
- Exclusion from services or opportunities due to identity

Possible Indicators of Hate Crime

- Reluctance to go out or attend certain places
- Fearfulness around particular people or environments
- Repeated reports of abuse or threats
- Sudden withdrawal or low mood
- Signs of injury or damage to property without clear explanation
- Direct disclosure of being targeted because of personal identity

Safeguarding Note

Hate crime is often under-reported. It may be motivated by subtle prejudice as well as overt hostility. The impact on the individual's sense of safety and dignity must always be taken seriously.

Action Prompt

If a hate crime is suspected, report it through BYO safeguarding procedures and escalate to the DSL or DDSL. Record all relevant details accurately. Police involvement may be necessary where a criminal offence has occurred.

12. Radicalisation

Definition

Radicalisation is the process by which an individual comes to support terrorism or forms of extremism that lead to terrorism. It can involve grooming, ideological influence, or exploitation, and is considered a safeguarding concern under the Prevent duty.

Types of Radicalisation Risk



- Exposure to extremist propaganda (online or in person)
- Influencing by individuals or groups promoting extremist views
- Isolation from mainstream society or networks
- Endorsement of violence or hate to advance ideology
- Sudden behavioural or belief changes
- Use of extremist language, symbolism, or actions

Possible Indicators of Radicalisation

- Talking in support of extremist or terrorist ideologies
- Withdrawing from usual activities or peers
- Obsessive interest in conspiracy theories or grievance narratives
- Possessing extremist literature or materials
- Expressing intolerance or hatred toward other groups
- Increasingly secretive behaviour, especially online

Safeguarding Note

Radicalisation is often subtle and may appear as a gradual shift in world-view. It may coexist with vulnerability, exploitation, or unmet emotional needs. Recognising and responding early is key.

Action Prompt

Concerns about radicalisation must be reported to the DSL or DDSL through standard safeguarding channels. Where appropriate, the Prevent referral process will be initiated in collaboration with relevant statutory partners.

13. Fabricated or Induced Illness

Definition

Fabricated or Induced Illness (FII) involves a person deliberately causing or feigning physical or psychological symptoms in another individual, often for attention, control, or access to services. Though commonly associated with child protection, FII can also occur in adult settings, especially where care is involved.

Types of FII



- Exaggerating or fabricating symptoms
- Inducing illness through medication, poisoning, or restriction
- Interfering with medical tests or equipment
- Preventing access to healthcare professionals
- Insisting on unnecessary treatments or interventions

Possible Indicators of FII

- Frequent or unusual medical presentations with no clear cause
- Discrepancies between reported symptoms and clinical evidence
- Symptoms only occurring in presence of the carer
- Carer overly involved or controlling in medical care
- Refusal to leave the adult alone with professionals

Safeguarding Note

FII may be difficult to detect and requires careful, multi-agency assessment. Prioritise documentation, clinical opinion, and safeguarding escalation.

Action Prompt

If FII is suspected, report it urgently to the DSL or DDSL and record the concern comprehensively. Safeguarding partners, including health services, may need to be involved.

14. Cyber Abuse / Online Exploitation

Definition

Cyber abuse or online exploitation involves the use of technology to harm, control, deceive, or exploit an individual. This may include grooming, blackmail, cyberstalking, identity theft, and distribution of explicit or abusive content.

Types of Cyber Abuse



- Online grooming or sexual exploitation
- Cyberbullying or repeated harassment
- Coercive control via messaging, tracking, or online threats
- Sharing of non-consensual images or videos
- Phishing, scams, or identity fraud for exploitation

Possible Indicators

- Withdrawal, anxiety, or fear around using devices
- Excessive secrecy or distress after internet use
- Reluctance to share online activity with trusted adults
- Sudden changes in social media use or behaviour
- Receiving gifts, money, or messages from unknown sources

Safeguarding Note

Online abuse can occur quickly and be hidden. It may escalate without intervention and often coexists with other abuse types.

Action Prompt

Report any concerns about cyber exploitation to the DSL/DDSL. Preserve evidence where possible and seek guidance on specialist digital safeguarding procedures.

15. County Lines / Criminal Exploitation

Definition

County lines exploitation refers to the grooming and coercion of vulnerable individuals into criminal activity, particularly drug trafficking, by gangs or organised networks. It involves manipulation, threats, and violence, and can affect adults as well as young people.

Types of Exploitation



- Transporting or storing drugs or weapons
- Being forced to deal or collect debts
- Living in unsafe or unfamiliar accommodation ('cuckooing')
- Grooming through gifts, debt, or intimidation
- Threats to self or family used to ensure compliance

Possible Indicators

- Repeated absence or going missing
- Possession of multiple phones or unexplained items
- Sudden change in lifestyle or income
- Appearing fearful, withdrawn, or secretive
- Being found in areas they have no connection to
- Known association with gangs or organised groups

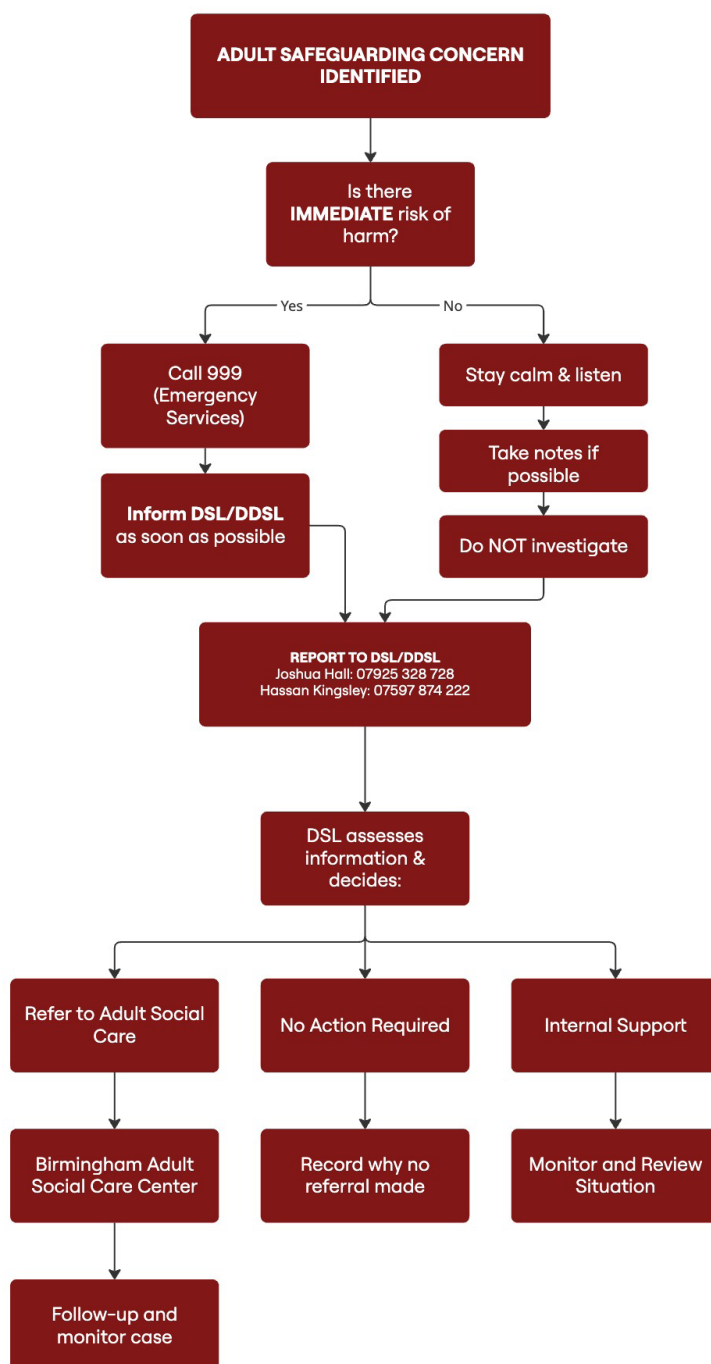
Safeguarding Note

County lines is a form of modern slavery and criminal exploitation. Victims may not identify as such and may appear complicit. A safeguarding-first approach is vital.

Action Prompt

If criminal exploitation is suspected, refer immediately to the DSL or DDSL. This must be treated as both a safeguarding and criminal matter, requiring urgent multi-agency response.

Appendix 2 - Adult Safeguarding Reporting Flowchart



SPECIAL CASES:

- Staff/Volunteer allegation → LADO (0121 675 1669)
- Historical abuse → Same process as current abuse
- Radicalisation → DSL contacts Prevent team
- No consent from adult → DSL considers public interest

ALTERNATIVE REPORTING:

- If unable to report through normal channels, see **WHISTLEBLOWING POLICY**

Document Control:

- This policy should be read in conjunction with our Safeguarding Policy Framework
- For concerns about children under 18, refer to Child Protection Procedures
- For allegations about staff conduct, refer to Managing Allegations about Staff and Volunteers Policy
- If unable to report through normal channels, see Whistleblowing Policy

Reviewed by:



Hassan Kingsley

Date: 28/06/2026

Next Review Date: 01/07/2027

This policy forms part of BYO's commitment to safeguarding and should be read alongside our complete Safeguarding Framework.

*Britannia Youth Organisation CIC is a Community Interest Company registered in England and Wales.
Company No. 12515346 | Registered Address: 36 St Joseph's Rd, Ward End, Birmingham, B8 2JU*



Managing Allegations about Staff and Volunteers

Part of the BYO Safeguarding and Protection Policy Suite

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Introduction

This guidance is part of Britannia Youth Organisation's (BYO) Safeguarding Framework as part of our commitment to keeping children, young people and adults at risk safe from abuse. It should be read in conjunction with:

- **Safeguarding Policy Framework**
- **Child Protection Procedures**
- **Adult Safeguarding Procedures**
- **Code of Conduct Policy**
- **Disciplinary Policy** (when developed)
- **Whistleblowing Policy**
- **Complaints Policy**
- **Recruitment, Selection and Onboarding Policy**

Framework Integration: This policy is a crucial component of our comprehensive safeguarding framework and complements procedures for managing allegations between young people outlined in our **Managing Allegations of Peer-to-Peer Abuse** policy.

1. Purpose & Scope

These procedures set out how to manage allegations (including safeguarding concerns) about BYO staff or volunteers and apply where allegations arise in relation to the staff or volunteer member:

- Carrying out duties for BYO; and
- Their conduct or behaviour outside of work or volunteering with BYO that may impact their suitability to work with children, young people or adults at risk

BYO will take into account information received from internal and external sources.

Who This Applies To

This policy covers allegations against:

- **Staff:** All employees (full-time, part-time, temporary, contract)
- **Volunteers:** All individuals giving unpaid time to BYO
- **Directors:** Board members and trustees
- **Contractors:** Third-party individuals providing services
- **Partners:** Representatives of partner organisations working with BYO

Recruitment Standards: All individuals covered by this policy undergo appropriate vetting as detailed in our **Recruitment, Selection and Onboarding Policy** and **Vetting and DBS Policy**.

2. What is an Allegation

An allegation concerns a staff or volunteer's behaviour, specifically that they have intentionally or unintentionally abused, exploited or put at risk, a child or 'adult at risk'.

Criteria for Allegations

If you are concerned that a member of staff or volunteer who works or volunteers for BYO has:

1. Behaved in a way that has harmed or may harm a child, young person or adult at risk of abuse
2. Possibly committed a criminal offence against or related to a child, young person or adult at risk of abuse
3. Behaved in a way that indicates he/she is unsuitable to work with children, young people or adults at risk of abuse
4. Behaved towards a child or children in a way that indicates they may pose a risk of harm to children

Types of Allegations

Allegations may include:

- **Physical abuse** - causing physical harm or injury
- **Sexual abuse** - inappropriate sexual conduct or exploitation
- **Emotional abuse** - psychological harm or inappropriate relationships
- **Neglect** - failure to provide appropriate care or supervision
- **Financial abuse** - misuse of position for financial gain
- **Grooming** - inappropriate relationship building or boundary crossing
- **Professional misconduct** - breach of professional standards or codes
- **Criminal activity** - any criminal behaviour, whether related to work or not

Code of Conduct: Expected standards of behaviour are outlined in our Code of Conduct Policy.

3. Reporting Contact Details

Primary Contacts

Designated Safeguarding Lead (DSL): Hassan Alexander Kingsley

- Mobile: 07597 874 222
- Email: hassankingsley@britanniayo.com

Deputy Designated Safeguarding Lead (DDSL): To be appointed

Alternative Reporting Routes

If the allegation concerns the DSL or DDSL:

- Contact the Chair of the Board of Directors directly
- Use external reporting channels (LADO, police, local authority)
- Follow **Whistleblowing Policy** procedures for protected disclosure

For anonymous or difficult reports:

- **Whistleblowing Policy** provides alternative reporting channels
- **NSPCC Whistleblowing Helpline:** 0800 028 0285
- **Local Authority Designated Officer (LADO):** 0121 675 1669

Alternative Reporting: If you feel unable to report through normal channels, see our **Whistleblowing Policy** for alternative options including anonymous reporting.

4. Initial Response and Documentation

Urgent Action Required

It is very important that you take action about your concerns without delay so as to prevent problems escalating and reduce the risk of harm. BYO's paramount concern is to safeguard children, young people and adults at risk.

Immediate Documentation Requirements

All allegations against staff or volunteers must be documented using the BYO Safeguarding Case Report Form immediately upon receipt. This ensures:

- **Comprehensive evidence gathering** from the outset
- **Legal admissibility** of documentation in formal proceedings
- **Consistency** in allegation management and investigation
- **Professional standard** of record-keeping for LADO and external agencies

When to Complete Safeguarding Case Report Form for Staff Allegations

The form must be completed for all allegations involving staff or volunteers who have:

1. **Behaved in a way that has harmed** or may harm a child, young person or adult at risk
2. **Possibly committed a criminal offence** against or related to a child, young person or adult at risk
3. **Behaved in a way that indicates** they are unsuitable to work with children, young people or adults at risk
4. **Behaved towards a child or children** in a way that indicates they may pose a risk of harm

Additional circumstances requiring the form:

- **Professional misconduct** involving safeguarding implications
- **Breach of safeguarding policies** or Code of Conduct with potential harm
- **Inappropriate relationships** with service users
- **Criminal activity** whether or not directly related to work
- **Concerning behaviour** outside of work that affects suitability

5. First Steps - Documentation Protocol

1. **Ensure immediate safety** - if someone is at immediate risk, call 999
2. **Complete Safeguarding Case Report Form** immediately (within 2 hours of allegation)
3. **Report to DSL/DDSL** with completed form within same time-frame
4. **Do not investigate** - avoid questioning the person making the allegation or the person alleged against
5. **Preserve evidence** - secure all relevant documentation, devices, communications
6. **Maintain confidentiality** - only share information with those who need to know

Specific Information Required for Staff Allegations

The Safeguarding Case Report Form must include:

Allegation Details

- **Specific nature** of the allegation with exact details
- **Date, time, location** of alleged incident(s)
- **Witnesses present** or potential witnesses
- **Evidence available** including documents, communications, CCTV
- **Impact on alleged victim(s)** - immediate and ongoing concerns

Staff Member Information

- **Full name and role** of person subject to allegation
- **Length of service** and previous positions
- **DBS check status** and any previous concerns
- **Training record** including safeguarding and professional development
- **Previous allegations** or disciplinary issues (if any)
- **Current duties** and access to children/vulnerable adults

Reporting Person Details

- **Name and role** of person making the allegation
- **Relationship to alleged victim** (if applicable)
- **How the allegation** came to their attention
- **Any concerns about** their motivation or credibility
- **Support needs** for the reporting person

Immediate Risk Assessment

- **Ongoing risk** to children or adults at risk
- **Access to potential victims** in current role
- **Suitability for continued** employment or volunteering
- **Need for suspension** or alternative duties
- **External agency involvement** required

If the Allegation Involves Criminal Activity

- **Call 999** if there is immediate danger
- **Call 101** for non-emergency police matters
- **Do not investigate** or gather evidence yourself
- **Follow police guidance** on preserving evidence
- **Inform LADO immediately** on 0121 675 1669

6. Investigation Process Documentation

Safeguarding Case Report Form in Investigations

The completed form serves as the foundation for all investigation activities:

LADO Reporting

- **Form submitted** to LADO within 1 working day alongside initial report
- **Provides comprehensive background** for LADO decision-making
- **Updated with LADO** recommendations and guidance
- **Used to track LADO** involvement and advice throughout process

Police Investigation Coordination

- **Shared with police** where criminal investigation required
- **Provides baseline** evidence for criminal proceedings
- **Updated with** police investigation progress and outcomes
- **Maintained separately** from internal investigation documentation

Internal Investigation Management

- **Forms basis** for internal investigation terms of reference
- **Updated with** investigation findings and evidence
- **Records all** witness statements and documentary evidence
- **Documents** investigation conclusions and recommendations

Ongoing Case Management Using the Form

The DSL will maintain the Safeguarding Case Report Form as a live document:

- **Regular updates** with new information or developments
- **Case conference notes** and multi-agency meeting outcomes
- **Risk assessment reviews** and any changes in circumstances
- **Support provided** to all parties involved
- **External agency communications** and responses
- **Timeline tracking** of all significant events and decisions

7. What Will Happen on Receipt of Allegation

Initial Response (Within 1 Working Day)

The DSL/DDSL will:

- **Acknowledge receipt** and review the information within one working day
- **Treat with confidentiality** throughout the process
- **Log in secure safeguarding records** with details of actions taken
- **Determine level of risk** and any immediate action required
- **Contact LADO** within 1 working day for guidance

Initial Risk Assessment

The DSL/DDSL will consider:

- **Ongoing risk** to children or adults at risk
- **Whether to report to police** for potential criminal activity
- **Securing evidence** for investigation (devices, laptops, computers, logs)
- **Welfare and well-being** of those making the allegation
- **Welfare and well-being** of the person against whom the allegation has been made
- **Requirements to report** allegations (funders, LADO, regulatory bodies)
- **Communications** with service users and other stakeholders

Immediate Safeguarding Actions

May include:

- **Suspension** from duties pending investigation
- **Alternative duties** away from direct contact with service users
- **Supervised contact** only with children/adults at risk
- **No contact** with BYO premises or activities
- **Reporting to external agencies** (police, LADO, regulatory bodies)

8. Decision-Making Process

Safeguarding Review Panel

If it is not clear whether the allegation meets the criteria for these procedures, advice and guidance will be sought through BYO's **Safeguarding Review Panel** consisting of:

- The DSL (or DDSL if DSL is subject of allegation)
- Chair of Board of Directors
- One independent Board member
- External safeguarding advisor (when appropriate)

LADO Consultation

All allegations must be reported to the Local Authority Designated Officer (LADO) within 1 working day.

LADO Contact Details:

- **Phone:** 0121 675 1669
- **Email:** lado.secure@birmingham.gcsx.gov.uk

The LADO will:

- Provide advice and guidance on next steps
- Coordinate with police if criminal investigation required
- Monitor progress of the investigation
- Advise on employment decisions

Board Oversight

The Board of Directors will:

- **Nominate oversight person(s)** for day-to-day management
- **Ensure appropriate support** is provided to all parties
- **Make employment decisions** based on investigation findings
- **Review policies and procedures** based on lessons learned

9. Investigation Process

Types of Investigation

Police Investigation:

- For potential criminal offences
- Led by police with LADO oversight
- BYO internal investigation may be delayed pending criminal investigation

Internal Investigation:

- For professional conduct and suitability matters
- Led by independent, appropriately trained investigator
- Follows fair and transparent process
- Results in recommendations for action

Independent Investigation

An internal investigation must be undertaken by:

- **Independent investigator** - not involved in day-to-day operations
- **Appropriately trained** - understands safeguarding and employment law
- **Competent professional** - with relevant experience and qualifications

The investigator will:

- **Gather facts** objectively and thoroughly
- **Interview relevant parties** appropriately
- **Review evidence** including documents and communications
- **Make recommendations** for action based on findings

Parallel Processes

These procedures may run concurrently with:

- **Disciplinary procedures** following internal investigation
- **Complaints procedures** if service users have made complaints
- **Whistleblowing procedures** if concerns about organisational culture
- **Criminal proceedings** if police investigation results in charges

Safeguarding Priority: In all cases, safeguarding must be paramount and the safety of children and adults at risk the first consideration.

10. Supporting All Parties - Documentation Requirements

Comprehensive Support Documentation

All support provided to parties must be recorded in the Safeguarding Case Report Form:

Support for Alleged Victims

- **Immediate safety measures** implemented and reviewed
- **Professional support** arranged (counselling, advocacy, medical)
- **Educational/workplace** adjustments made
- **Family liaison** and communication arrangements
- **Long-term support planning** and review arrangements

Support for Person Making Allegation

- **Protection measures** against potential retaliation
- **Professional support** including counselling if traumatised
- **Workplace adjustments** or flexible arrangements
- **Regular check-ins** and welfare monitoring
- **Whistleblowing protection** measures implemented

Support for Person Subject to Allegation

- **Fair treatment measures** and due process protections
- **Professional support** including legal advice access
- **Workplace arrangements** during investigation (suspension, alternative duties)
- **Communication plan** for investigation progress updates
- **Welfare monitoring** and mental health support

Support for Wider Team

- **Team briefings** and reassurance (maintaining confidentiality)
- **Additional supervision** for staff affected by allegations
- **Professional support** for managers handling difficult situations
- **Service continuity** planning to minimise disruption

11. Special Considerations

Digital Evidence and Indecent Images

Important: BYO will not copy, print, share, store or save indecent images of children as this is illegal.

When allegations involve indecent images:

- **Do not view** or handle suspected illegal images
- **Secure devices** immediately and do not attempt to access content
- **Contact police immediately** for guidance on evidence handling
- **Protect staff** from traumatic exposure to illegal content
- **Follow police guidance** throughout the process

Historical Allegations

For allegations about past events:

- **Take seriously** regardless of when the alleged abuse occurred
- **Consider current risk** posed by the individual
- **Contact LADO** even for historical allegations
- **Support alleged victims** who may have carried trauma for years
- **Investigate thoroughly** as patterns of behaviour may exist

Multiple Allegations

When multiple allegations arise:

- **Consider patterns** of concerning behaviour
- **Coordinate investigations** to avoid duplication or conflict
- **Enhanced safeguarding measures** may be required
- **LADO involvement** becomes even more critical
- **Media interest** may need to be managed

12. Outcomes and Actions

Possible Investigation Outcomes

Substantiated: Evidence supports the allegation

False: Evidence disproves the allegation

Malicious: Allegation is deliberately invented or knowingly false

Unsubstantiated: Insufficient evidence either way

Unfounded: No evidence or concern about the person's behaviour

Employment Actions

Based on investigation findings:

- **No action required** - person continues in role
- **Additional training** - addressing identified development needs
- **Disciplinary action** - warnings, conditions, or restrictions
- **Dismissal** - for gross misconduct or unsuitability
- **Referral to DBS** - for barring consideration
- **Professional body referral** - for professional misconduct

Regulatory Reporting

DBS Referral: Required when person is dismissed or would have been dismissed for harm or risk to children/adults at risk

Professional Bodies: Relevant professional registration bodies must be informed of serious misconduct

Regulatory Bodies: Charity Commission or other regulators may need to be informed

13. Communication and Media

Internal Communication

- **Staff and volunteers** - appropriate briefing about safety measures
- **Service users** - reassurance about ongoing safety and support
- **Board members** - regular updates on progress
- **Partner organisations** - information sharing as appropriate

External Communication

- **Funders** - notification as required by funding agreements
- **Media enquiries** - handled by designated spokesperson only
- **Community concerns** - addressed through appropriate channels
- **Legal advisers** - consultation on complex cases

Confidentiality

- **Need to know basis** - information shared only with those requiring it
- **Data protection** - compliance with Data Protection Policy
- **Legal constraints** - police investigations may limit information sharing
- **Reputation management** - balanced with transparency obligations

14. Records and Information Management

Safeguarding Case Report Form as Master Record

The completed Safeguarding Case Report Form serves as the master record for all allegation cases and must contain:

Comprehensive Case History

- **Initial allegation details** and all subsequent developments
- **Investigation timeline** with key dates and milestones
- **All agency involvement** including LADO, police, regulatory bodies
- **Decision-making processes** and rationale for key decisions
- **Outcome documentation** including disciplinary actions or exoneration

Evidence Management

- **Inventory of all evidence** collected during investigation
- **Witness statement summaries** and interview notes
- **Documentary evidence** references and secure storage locations
- **Digital evidence** handling and forensic preservation records
- **Chain of custody** documentation for all physical evidence

Multi-Agency Coordination

- **LADO communications** and advice received
- **Police liaison** and investigation coordination
- **Regulatory body** notifications and responses
- **Professional body** referrals and outcomes
- **DBS referral** decisions and processing

Storage and Access Control

The Safeguarding Case Report Form and associated documentation:

- **Stored in secure** safeguarding filing system with restricted access
- **Electronic copies** maintained on password-protected systems only
- **Access limited** to DSL, DDSL, investigating officers, and authorised personnel
- **Audit trail maintained** of all access and file modifications
- **Retention schedule** applied in accordance with legal requirements and Data Protection Policy

Information Sharing Protocols

Form information may be shared with:

- **LADO** for case management and oversight
- **Police** for criminal investigation purposes
- **DBS** for barring and suitability decisions
- **Professional regulatory bodies** for fitness to practice assessments
- **Employment tribunals** or legal proceedings as required
- **Insurance companies** for liability and coverage decisions

All sharing must be:

- **Documented** in the case report form with dates, recipients, and purposes
- **Proportionate** to the legitimate need for information
- **Consistent** with data protection requirements and confidentiality obligations
- **Authorised** by appropriate senior personnel

15. Training Requirements for Staff Allegation Documentation

Specialised Training for Allegation Management

All managers and DSL personnel must receive training on:

Form Completion for Staff Allegations

- **Legal requirements** for documentation in employment and criminal proceedings
- **Evidence standards** and admissibility in formal investigations
- **Confidentiality obligations** and information sharing protocols
- **LADO reporting** requirements and coordination procedures

Investigation Management

- **Maintaining objectivity** and avoiding bias in documentation
- **Evidence preservation** and chain of custody requirements
- **Witness interview** techniques and statement recording
- **Multi-agency coordination** and information sharing protocols

Legal and Professional Considerations

- **Employment law** implications of allegation documentation
- **Criminal procedure** requirements for evidence handling
- **Professional regulation** and fitness to practice referrals
- **Data protection** and human rights considerations

Case Management Skills

- **Risk assessment** and ongoing safety planning
- **Support coordination** for all parties involved
- **Communication management** with stakeholders and agencies
- **Quality assurance** and case review procedures

Annual refresher training will include case study analysis using completed Safeguarding Case Report Forms to demonstrate best practice in allegation documentation and management.

16. Learning and Improvement

Post-Incident Review

Once matters are concluded, BYO will:

- **Review what happened** and identify lessons learned
- **Update policies** and procedures as appropriate
- **Enhance training** programmes based on findings
- **Improve practices** to prevent future incidents
- **Share learning** with relevant stakeholders

Organisational Development

Learning may result in:

- **Policy updates** across the safeguarding framework
- **Enhanced training** for staff and volunteers
- **Improved recruitment** and vetting procedures
- **Better supervision** and support systems
- **Cultural changes** to promote safeguarding

Related Policies

This policy should be read alongside:

- **Safeguarding Policy Framework**
- **Child Protection Procedures**
- **Adult Safeguarding Procedures**
- **Managing Allegations of Peer-to-Peer Abuse**
- **Code of Conduct Policy**
- **Whistleblowing Policy**
- **Complaints Policy**
- **Recruitment, Selection and Onboarding Policy**
- **Vetting and DBS Policy**
- **Data Protection Policy**
- **Anti-Bribery and Corruption Policy**

17. Key External Contacts

Statutory Agencies

Local Authority Designated Officer (LADO): 0121 675 1669 **Police Emergency:** 999 **Police Non-Emergency:** 101 **Birmingham Children's Services:** 0121 303 1888 **Birmingham Adult Social Care:** 0121 303 1234

Support Organisations

NSPCC Whistleblowing Helpline: 0800 028 0285 **Disclosure and Barring Service:** 03000 200 190
ACAS Employment Relations: 0300 123 1100

Document Control

- This policy forms part of BYO's comprehensive safeguarding framework
- All staff receive training on recognising and reporting concerns about colleagues
- Regular reviews ensure procedures remain effective and compliant

This framework should be read as the foundation document alongside all other BYO safeguarding policies and procedures. For any queries about this framework, please contact the Designated Safeguarding Lead.

This policy has been approved by Hassan Kingsley, the Governor, and will be reviewed annually or following any significant incident or change in legislation.

Reviewed by:



Hassan Kingsley
Governor

Date: 28/06/2026

Next Review Date: 01/07/2027

This policy forms part of BYO's commitment to safeguarding and should be read alongside our complete Safeguarding Framework.

*Britannia Youth Organisation CIC is a Community Interest Company registered in England and Wales.
Company No. 12515346 | Registered Address: 36 St Joseph's Rd, Ward End, Birmingham, B8 2JU*



Managing Allegations of Peer-to-Peer Abuse

Part of the BYO Safeguarding and Protection Policy Suite

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Document Details	Information
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Cross-References	Safeguarding Policy Framework, Child Protection Procedures, Adult Safeguarding Procedures, Code of Conduct Policy, Whistleblowing Policy, Complaints Policy, Recruitment, Selection and Onboarding Policy, Anti-Bribery and Corruption Policy

Introduction

This guidance is part of Britannia Youth Organisation's (BYO) Safeguarding Framework as part of our commitment to keeping children, young people and adults at risk safe from abuse. It should be read in conjunction with:

- Safeguarding Policy Framework
- Child Protection Procedures
- Adult Safeguarding Procedures
- Managing Allegations about Staff and Volunteers
- Code of Behaviour Policy
- Anti-Bullying Policy
- Digital Safeguarding Policy
- Whistleblowing Policy
- Complaints Policy

Framework Integration: This policy complements procedures for managing allegations against staff and volunteers outlined in our **Managing Allegations about Staff and Volunteers policy** and works alongside general behaviour management through our **Code of Behaviour Policy**.

1. Purpose & Scope

These procedures set out how to manage allegations of peer-to-peer abuse between BYO service users, including children, young people, and adults at risk. This policy applies to incidents that occur:

- During BYO activities and programmes
- During BYO-organised trips or events
- Involving BYO service users outside of direct BYO activities where there may be ongoing safeguarding implications
- In digital spaces related to BYO activities

BYO recognises that peer-to-peer abuse can take many forms and will take into account information received from internal and external sources.

Important Distinctions

- **Peer-to-peer abuse** (covered by this policy) - serious harmful behaviour between service users
- **General behaviour management** - covered by **Code of Behaviour Policy**
- **Bullying** - addressed through **Anti-Bullying Policy** (may escalate to this policy)
- **Staff/volunteer concerns** - covered by **Managing Allegations about Staff and Volunteers**

2. What is Peer-to-Peer Abuse

Peer-to-peer abuse occurs when a child, young person, or adult at risk is abused by another child, young person, or adult at risk. This includes but is not limited to:

Types of Peer-to-Peer Abuse

Physical abuse: Hitting, kicking, shaking, biting, hair pulling, or otherwise causing physical harm

Sexual abuse: Inappropriate sexual behaviour, sexual harassment, sexual assault, or exploitation

Emotional abuse: Persistent emotional maltreatment, humiliation, or intimidation

Online abuse: Cyberbullying, sharing inappropriate images, online harassment, or exploitation through digital platforms

Financial abuse: Theft, fraud, or exploitation of money or possessions

Discriminatory abuse: Abuse based on protected characteristics including race, gender, disability, sexual orientation, or religion

Harmful Sexual Behaviour

Harmful Sexual Behaviour requires particular attention and includes:

- Age-inappropriate sexual knowledge or behaviour
- Problematic or concerning sexual behaviour
- Sexually harmful behaviour that may constitute a criminal offence
- Sharing of sexual images or content
- Sexual exploitation or coercion

Digital Considerations: Online peer-to-peer abuse is addressed through our **Digital Safeguarding Policy** alongside this policy.

Bullying vs. Abuse: Serious or persistent bullying may escalate to peer-to-peer abuse. See our **Anti-Bullying Policy** for initial response to bullying behaviours.

3. Recognition and Response

Immediate Documentation Requirements for Peer-to-Peer Incidents

All peer-to-peer abuse incidents must be documented using the **BYO Safeguarding Case Report Form** immediately upon identification. This ensures:

- **Comprehensive evidence** collection for potential police investigation
- **Professional standard** documentation for multi-agency working
- **Legal protection** for the organisation and appropriate response
- **Consistency** in handling serious peer-to-peer incidents

When to Complete Safeguarding Case Report Form for Peer-to-Peer Incidents

The form must be completed for all incidents where:

1. **A service user has been harmed** or may be harmed by another service user
2. **There are concerns about sexually harmful behaviour** between service users
3. **A service user has possibly committed** a criminal offence against another service user
4. **Behaviour indicates ongoing risk** to other service users or the wider community
5. **Serious bullying** that involves power imbalances or potential for significant harm
6. **Digital abuse** including cyberbullying, sharing inappropriate images, or online exploitation
7. **Financial exploitation** between service users
8. **Discriminatory abuse** based on protected characteristics

Distinguished from general behaviour management:

- **Minor behavioural incidents** managed through Code of Behaviour Policy do not require the form
- **Low-level bullying** addressed through Anti-Bullying Policy may not require the form initially
- **When in doubt**, complete the Safeguarding Case Report Form - the DSL can determine appropriateness

Reporting Contact Details

Primary Contacts

Designated Safeguarding Lead (DSL): Hassan Alexander Kingsley

- Mobile: 07597 874 222
- Email: hassankingsley@britanniayo.com

Deputy Designated Safeguarding Lead (DDSL): To be appointed

Emergency Situations

Emergency: If there is immediate risk of harm, contact 999 first, then notify the DSL/DDSL immediately.

Non-emergency police: 101 for criminal matters requiring police involvement

Alternative Reporting

If unable to report directly:

- Use Whistleblowing Policy procedures for alternative reporting channels
- Contact Childline: 0800 1111 for young people needing support
- Contact NSPCC: 0808 800 5000 for professional advice

Alternative Reporting: If you feel unable to report through normal channels, see our Whistleblowing Policy for alternative options including anonymous reporting.

4. Initial Response and Risk Assessment

Immediate Documentation Protocol

Contact immediately and complete **Safeguarding Case Report Form** if:

- Someone is at immediate risk of harm
- There are signs of serious physical injury
- Sexual abuse is suspected or disclosed
- Criminal activity is suspected
- There is risk of further incidents

Specific Information Required for Peer-to-Peer Incidents

The **Safeguarding Case Report Form** must include:

Incident Details

- **Exact description** of what happened using witness accounts
- **Location and timing** of the incident(s)
- **Whether incident was** witnessed by staff, other young people, or reported later
- **Any ongoing pattern** of concerning behaviour between the individuals
- **Power dynamics** between the individuals involved (age, size, developmental differences)

Alleged Perpetrator Information

- **Age and developmental** level
- **Previous concerning behaviour** or safeguarding issues
- **Current support needs** and vulnerabilities
- **Family circumstances** and potential contributing factors
- **Understanding of** appropriate behaviour and boundaries
- **Risk assessment** for continued participation in activities

Alleged Victim Information

- **Age and vulnerability** factors
- **Impact of incident** - physical, emotional, psychological
- **Previous victimisation** or safeguarding concerns
- **Support needs** and protective factors
- **Views and wishes** about reporting and intervention
- **Safety planning** requirements

Environmental Factors

- **Supervision arrangements** at time of incident
- **How incident came** to staff attention
- **Other service users** who may have witnessed or been affected
- **Venue or activity** specific risk factors
- **Time of day** and staffing levels

Decision-Making Process

Safeguarding Review Panel

If it is not clear whether the allegation meets the criteria for peer-to-peer abuse, advice and guidance must be sought through **BYO's Safeguarding Review Panel** consisting of:

- The DSL (or DDSL if necessary)
- Chair of Board of Directors
- One independent Board member
- External safeguarding advisor (when appropriate)

Immediate Safety Planning

The DSL will prioritise:

1. **Safety planning:** Immediate steps to protect all service users
2. **Appropriate supervision:** Ensuring safe supervision arrangements
3. **Separation if necessary:** Preventing further contact between parties if required
4. **Evidence preservation:** Securing physical and digital evidence
5. **Professional consultation:** Seeking advice from Children's Services, LADO, or police as appropriate

5. External Agency Involvement

When to Involve External Agencies

The DSL will determine when to involve:

Children's Services: For all allegations involving children and young people

- Contact: Birmingham Children's Services MASH - 0121 303 1888

Adult Services: Where adults at risk are involved

- Contact: Birmingham Adult Social Care - 0121 303 1234

Police: Where criminal offences may have been committed

- Contact: 999 (emergency) or 101 (non-emergency)

LADO: Where the allegation involves organised activities or where staff response is in question

- Contact: 0121 675 1669

Multi-Agency Working

BYO will work collaboratively with:

- Children's Services
- Police (where criminal investigation required)
- Educational establishments
- Health services
- Youth Offending Team (if applicable)
- Other relevant agencies

6. Parental/Carer Notification

When to Inform Parents/Carers

Parents/carers of all children and young people involved will normally be informed at the earliest opportunity, unless:

- It would place the child at additional risk
- It would compromise a police investigation
- Professional advice recommends otherwise
- Safeguarding concerns exist about the home environment

Recording Parental Communication in Safeguarding Case Report Form

All parental/carers communication must be documented including:

Notification Planning

- **Decision-making process** about when and how to inform parents/carers
- **Consultation with** Children's Services or police about timing
- **Risk assessment** of parental reaction and impact on children
- **Coordination between** families to prevent conflict or retaliation

Communication Records

- **Date, time, method** of communication with each family
- **Who was present** during conversations
- **Information shared** and any limitations on disclosure
- **Parental reactions** and responses to incident disclosure
- **Support offered** to families and children
- **Follow-up arrangements** and ongoing communication plans

Consent and Cooperation

- **Consent given** for investigation and multi-agency involvement
- **Any objections** or concerns raised by parents/carers
- **Cooperation level** with safeguarding processes
- **Additional support** needs identified for families
- **Conflict between** families and mediation requirements

Safeguarding Priority: Child protection considerations take precedence over parental rights to information.

7. Investigation and Management

Safeguarding Case Report Form in Peer-to-Peer Investigations

The completed form serves multiple investigation purposes:

Multi-Agency Coordination

- **Children's Services referral** supported by comprehensive form documentation
- **Police liaison** where criminal activity is suspected
- **LADO consultation** where organisational response is questioned
- **School coordination** where incidents affect education settings
- **Health services** involvement for therapeutic support

Internal Investigation Management

- **Evidence gathering** and witness statement coordination
- **Risk assessment** development and ongoing review
- **Safety planning** for all service users involved
- **Parental communication** and consent management
- **Service delivery** adjustments and supervision enhancement

Ongoing Case Management Using the Form

The DSL will maintain the form as a live document throughout:

Investigation Progress

- **External agency** involvement and case reference numbers
- **Investigation findings** and evidence gathered
- **Professional meetings** and multi-agency discussions
- **Risk assessment** updates and safety plan modifications
- **Outcome decisions** and intervention planning

Support and Intervention

- **Therapeutic support** arranged for all parties
- **Educational interventions** about appropriate behaviour
- **Family work** and parental support
- **Restorative approaches** where appropriate and safe
- **Ongoing monitoring** and progress review

Internal Investigation

An internal investigation will be conducted by an appropriately trained professional who will:

- **Gather facts and evidence** objectively and thoroughly
- **Interview relevant parties** with appropriate support and safeguards
- **Review policies and procedures** to identify any systemic issues
- **Make recommendations** for action and improvement
- **Coordinate with external investigations** where applicable

Investigation Principles

Fair process: All parties treated fairly and with dignity

Child-focused: Best interests of children paramount

Proportionate: Response appropriate to the severity of allegations

Timely: Investigations completed without unnecessary delay

Thorough: All relevant evidence considered Documented: Clear records maintained throughout

Ongoing Risk Management

Throughout the process, BYO will:

- **Regularly review and update** risk assessments
- **Implement appropriate safety measures** for all service users
- **Monitor the well-being** of all parties involved
- **Adjust service delivery** as necessary to maintain safety
- **Coordinate with external agencies** on ongoing risk management

8. Special Considerations for Different Types of Peer-to-Peer Abuse

Harmful Sexual Behaviour Documentation

Requires enhanced documentation in the Safeguarding Case Report Form:

- **Detailed behavioural** description without unnecessary graphic detail
- **Age-appropriateness** assessment of sexual knowledge or behaviour
- **Consent and capacity** considerations for all parties
- **Previous sexual behaviour** concerns or sexualised behaviour patterns
- **Access to inappropriate** sexual content or exposure to abuse
- **Assessment of understanding** about appropriate sexual behaviour
- **Specialist consultation** with sexual behaviour specialists or CAMHS
- **Criminal justice** considerations and police liaison requirements

Digital Peer-to-Peer Abuse Documentation

Special requirements for online incidents:

- **Digital evidence** preservation including screen-shots, messages, posts
- **Platform involvement** and reporting to social media companies
- **Technical details** of how abuse was perpetrated online
- **Reach and impact** of digital abuse (how many people saw content)
- **Device security** and access controls
- **Digital footprint** considerations and ongoing online presence
- **Education needs** about digital citizenship and online safety

Physical Violence Documentation

Comprehensive recording requirements:

- **Injury assessment** and medical treatment provided
- **Witness accounts** from multiple perspectives where available
- **Weapon or object** involvement in violence
- **Precipitating factors** and build-up to violence
- **Self-defence considerations** and provocation assessment
- **Pattern of aggressive** behaviour or escalation over time
- **Risk of retaliation** or ongoing conflict between parties

9. Support and Welfare

For Alleged Victims

- **Immediate safety** and protection measures
- **Emotional support** and counselling referrals
- **Advocacy and representation** as needed
- **Regular welfare checks** and communication
- **Support to continue accessing** appropriate services
- **Long-term support** planning

For Alleged Perpetrators

- **Fair treatment** throughout the process
- **Appropriate support** recognising they may also be vulnerable
- **Access to specialist interventions** if required
- **Continued safeguarding** as a potentially vulnerable person
- **Educational support** about appropriate behaviour
- **Reintegration planning** where appropriate

For Witnesses and Other Service Users

- **Reassurance and support** about safety measures
- **Clear communication** about what is happening (age-appropriate)
- **Continued access** to services where safe and appropriate
- **Additional support** if traumatised by events

For Parents/Carers

- **Regular communication** and updates
- **Support and guidance** on managing the situation
- **Signposting to additional** support services
- **Information about** their rights and the process

For Staff and Volunteers

- **Support for those** managing difficult situations
- **Debriefing sessions** after serious incidents
- **Additional training** if knowledge gaps identified
- **Counselling support** if traumatised by events

10. Outcomes and Interventions

Possible Outcomes

Following investigation, BYO may implement various measures including:

No further action: Where allegations are unsubstantiated

Restorative approaches: Where appropriate and safe for all parties

Additional support: Enhanced support packages for those involved

Educational interventions: Training on appropriate behaviour and relationships

Restricted access: Modified participation in activities with additional supervision

Suspended access: Temporary suspension from services pending further assessment

Permanent exclusion: Removal from services where ongoing risk cannot be managed

Referral to specialist services: For assessment and intervention

Criminal justice involvement: Where criminal offences have occurred

Decision-Making Criteria

All decisions will be:

- **Proportionate** to the findings and risk assessment
- **Focused on safeguarding** and welfare of all parties
- **Regularly reviewed** and updated as circumstances change
- **Clearly communicated** to relevant parties
- **Documented** with clear rationale

Appeals Process: Decisions can be challenged through our **Complaints Policy** procedures.

11. Digital and Online Considerations

Online Peer-to-Peer Abuse

Special considerations for digital incidents:

- **Evidence preservation** - screen-shots, messages, posts
- **Platform reporting** - reporting to social media platforms
- **Device security** - securing devices with evidence
- **Digital footprint** - considering ongoing online presence
- **Education** about digital citizenship and online safety

Digital Safety: All online peer-to-peer abuse incidents are also addressed through our **Digital Safeguarding Policy**.

Sharing of Images

Particular attention required for:

- **Sharing of sexual images** - criminal implications
- **Humiliating images** - impact on dignity and well-being
- **Identifying information** - privacy and safety concerns
- **Viral content** - difficulty in controlling spread

12. Record Keeping and Information Management

Peer-to-Peer Safeguarding Case Report Form Management

The completed form serves as the comprehensive record for:

Legal and Criminal Proceedings

- **Evidence base** for potential criminal prosecution
- **Professional witness** statements and expert assessments
- **Timeline documentation** for court proceedings
- **Multi-agency** coordination and information sharing
- **Victim impact** assessments and ongoing harm evaluation

Educational and Therapeutic Planning

- **Intervention planning** based on documented needs and risks
- **Progress monitoring** and outcome evaluation
- **Specialist referrals** and professional involvement
- **Safety planning** and risk management strategies
- **Restorative justice** considerations where appropriate

Organisational Learning

- **Policy development** and procedure improvement
- **Staff training** needs identification
- **Risk assessment** enhancement for activities and environments
- **Preventive measures** and early intervention strategies
- **Quality assurance** and safeguarding effectiveness review

Documentation Requirements

Detailed records must be maintained including:

- **Initial allegation** and circumstances
- **All actions taken** and by whom
- **Dates and details** of all communications
- **Risk assessments** and reviews
- **Investigation findings** and decisions
- **Ongoing monitoring** arrangements
- **Support provided** to all parties

Storage and Access

- **Secure safeguarding filing system** with restricted access
- **Confidentiality protection** for all parties
- **Data protection compliance** as per **Data Protection Policy**
- **Retention schedules** for different types of records
- **Access rights** for those involved

Information Sharing for Peer-to-Peer Cases

Form information may be shared with:

- Children's Services for child protection assessment and intervention
- Police for criminal investigation where offences committed
- Youth Offending Team for diversion and intervention programs
- Educational settings for safety planning and support coordination
- Health services including CAMHS for therapeutic intervention
- Specialist services for harmful sexual behaviour or trauma support

Sharing protocols must ensure:

- Proportionate disclosure appropriate to each agency's role
- Consent considerations especially for older young people
- Confidentiality balance between parties involved
- Ongoing coordination between agencies without duplication
- Regular review of information sharing arrangements

13. Communication Management

Internal Communications

- **Staff briefings** about safety measures (without breaching confidentiality)
- **Board reporting** on serious incidents and lessons learned
- **Service user communications** about safety and support
- **Parent/carer updates** where appropriate

External Communications

Carefully managed to:

- **Protect privacy** and dignity of all involved
- **Prevent stigmatisation** of individuals or groups
- **Maintain confidence** in BYO's services
- **Comply with legal** requirements and restrictions

Media Management

- **Designated spokesperson** for any media enquiries
- **Legal advice** on complex or high-profile cases
- **Reputation management** balanced with transparency
- **Victim protection** from unwanted publicity

14. Training Requirements for Peer-to-Peer Abuse Documentation

Specialised Training for Peer-to-Peer Cases

All staff must receive training on:

Recognition and Documentation

- **Identifying peer-to-peer** abuse versus normal conflict or behaviour
- **Understanding developmental** factors affecting behaviour assessment
- **Documenting incidents** objectively without bias or assumption
- **Recognising complex** dynamics in peer relationships

Evidence Preservation

- **Digital evidence** collection and preservation
- **Physical evidence** protection including injury documentation
- **Witness statement** coordination and child-friendly approaches
- **Avoiding contamination** of evidence through inappropriate questioning

Multi-Agency Working

- **Children's Services** referral processes and coordination
- **Police liaison** for criminal investigation matters
- **Educational setting** communication and safety planning
- **Health service** coordination for therapeutic support

Supporting All Parties

- **Trauma-informed approaches** for both alleged victims and perpetrators
- **Family communication** and conflict management
- **Ongoing safety** planning and risk management
- **Restorative approaches** where appropriate and safe

Scenario-Based Training

Regular training includes:

- **Case study analysis** using anonymised Safeguarding Case Report Forms
- **Role-play exercises** in completing forms under pressure
- **Multi-agency simulation** exercises for complex cases
- **Legal requirement** updates and best practice sharing
- **Quality assurance** review of completed forms and learning opportunities

Staff and Volunteer Training

BYO ensures that:

- **All staff and volunteers** receive training on recognising and responding to peer-to-peer abuse
- **Specialist training** for those managing investigations
- **Regular updates** on best practice and legal requirements
- **Scenario-based training** to practice responses

Service User Education

- **Age-appropriate education** about healthy relationships and personal safety
- **Consent and boundaries** education
- **Digital citizenship** and online safety training
- **Bystander intervention** training
- **Reporting mechanisms** clearly explained

Parent/Carer Engagement

- **Information about** peer-to-peer abuse and prevention
- **Warning signs** to look out for
- **How to support** their child if affected
- **BYO's approach** to prevention and response

15. Learning and Development

Post-Incident Review

Following conclusion of each case, BYO will:

- Conduct a case review to identify learning opportunities
- Update policies and procedures as necessary
- Provide additional training where gaps are identified
- Share learning (anonymised) with the wider organisation
- Implement preventative measures to reduce future risks

Organisational Learning

May result in:

- Policy updates across the safeguarding framework
- Enhanced training programmes for staff and volunteers
- Improved risk assessment procedures
- Better support systems for service users
- Cultural changes to promote safety and respect

16. Quality Assurance and Monitoring

Peer-to-Peer Case Form Quality Review

The DSL conducts regular quality assurance including:

Form Completion Standards

- **Comprehensive information** gathering and documentation
- **Objective recording** without bias or inappropriate interpretation
- **Evidence quality** and admissibility considerations
- **Timeline accuracy** and chronological consistency
- **Multi-agency coordination** effectiveness

Outcome Monitoring

- **Investigation effectiveness** and evidence quality
- **Support provision** adequacy and impact
- **Safety planning success** and ongoing risk management
- **Learning application** from case outcomes
- **Prevention effectiveness** and early intervention impact

Continuous Improvement

- **Policy updates** based on case learning and outcomes
- **Training enhancement** addressing identified gaps or needs
- **Procedure refinement** for investigation and support coordination
- **Resource allocation** for effective case management
- **Partnership development** with external agencies and specialists

Effectiveness Monitoring

The effectiveness of this policy will be monitored through:

- **Regular review** of cases and outcomes
- **Feedback from** service users, parents/carers, and staff
- **Consultation with** external agencies and experts
- **Annual policy review** and update
- **Performance indicators** and trend analysis

Continuous Improvement

- **Learning from incidents** to prevent recurrence
- **Best practice research** and implementation
- **Stakeholder consultation** on policy effectiveness
- **External reviews** and inspections
- **Benchmarking** against other organisations

Annual review of all peer-to-peer Safeguarding Case Report Forms provides organisational learning and service improvement opportunities, ensuring that documentation standards continue to meet legal, professional, and safeguarding effectiveness requirements.

17. Related Policies

This policy should be read alongside:

- Safeguarding Policy Framework
- Child Protection Procedures
- Adult Safeguarding Procedures
- Managing Allegations about Staff and Volunteers
- Code of Behaviour Policy
- Anti-Bullying Policy
- Digital Safeguarding Policy
- Whistleblowing Policy
- Complaints Policy
- Data Protection Policy
- Equality, Diversity and Inclusion Policy

Policy Priority: Where multiple policies apply, safeguarding must be paramount and the safety of children and adults at risk the first consideration.

18. Key External Contacts

Statutory Agencies

Birmingham Children's Services MASH: 0121 303 1888 **Birmingham Adult Social Care:** 0121 303 1234
Police Emergency: 999 **Police Non-Emergency:** 101 **LADO:** 0121 675 1669

Support Services

Childline: 0800 1111 **NSPCC:** 0808 800 5000 **Samaritans:** 116 123 **Relate:** 0300 100 1234
(relationship counselling)

Document Control

- This policy forms part of BYO's comprehensive safeguarding framework
- All staff receive training on recognising and responding to peer-to-peer abuse
- Regular reviews ensure procedures remain effective and compliant

This framework should be read as the foundation document alongside all other BYO safeguarding policies and procedures. For any queries about this framework, please contact the Designated Safeguarding Lead.

This policy has been approved by Hassan Kingsley, the Governor, and will be reviewed annually or following any significant incident or change in legislation.

Reviewed by:



Hassan Kingsley

Governor

Date: 28/06/2026

Next Review Date: 01/07/2027

This policy forms part of BYO's commitment to safeguarding and should be read alongside our complete Safeguarding Framework.

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Implementation and Operational Procedures



Children and Young People Activities Standard Operating Procedure

Part of the BYO Safeguarding and Protection Policy Suite

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Document Details	Information
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Owner	Designated Safeguarding Lead
Approved By	Hassan Kingsley, Governor
Cross-References	Safeguarding Policy Framework, Child Protection Procedures, Digital Safeguarding Policy, Health and Safety Policy, Offsite Visits Policy, Code of Behaviour Policy, Data Protection Policy, Vetting and DBS Policy, Recruitment Selection and On-boarding Policy, Whistleblowing Policy, Complaints Policy

1. Purpose & Application

This procedure ensures safe delivery of all BYO activities for **children** (under 18) and young adults (18-24) including Sports Drive, Allotment Project, Homework Club, and trips.

Important: All BYO provision operates on an **open access basis** unless otherwise specified. Participants can come and go freely - we do not provide childcare and it is not staff's responsibility to ensure participants remain on-site.

Key Reference: When planning and delivering activities with adults, **staff** and **volunteers** must follow the BYO **Safeguarding Policy Framework** and our guidance on planning and running activities. BYO deliver services for young people which includes both children (under 18) and young adults (18-24).

2. Safeguarding Requirements

All staff and volunteers must:

- Follow BYO's **Safeguarding Policy Framework**
- Complete **safeguarding** training within 3 months of starting
- Hold Enhanced DBS clearance (renewed every 3 years)
- Report all **safeguarding** concerns to **Designated Safeguarding Lead** immediately

See also: *[Vetting and DBS Policy]* for detailed DBS requirements and *[Recruitment, Selection and Onboarding Policy]* for safer recruitment practices.

Designated Safeguarding Lead: Hassan Kingsley (07597 874 222, safeguarding@britanniayo.com)

Deputy Designated Safeguarding Lead: To be appointed (safeguarding@britanniayo.com)

Alternative Reporting Routes

If you feel unable to report concerns directly to the **DSL**:

- Report to the DDSL if concerns relate to the **DSL**
- Use **[Whistleblowing Policy]** procedures for serious organisational concerns
- Access **[Complaints Policy]** for service-related issues
- Contact external authorities directly if immediate risk is present

3. Staff ratios and Competency

For Open Access Sessions:

- **All ages:** 1:12 (flexible as participants come and go)
- **Minimum:** Always 2 staff members present

For Closed/Targeted Sessions & Trips:

- **Under 8s:** 1:6
- **8-17 years:** 1:8
- **18-24 years:** 1:10
- **Trips/offsite:** 1:6 (all ages)

Staff Competency Requirements:

- All **staff** must have completed **safeguarding** training (renewed every 3 years)
- At least one first aider present at each session
- Activity-specific qualifications as required
- Enhanced DBS checks for all roles

See also: *[Recruitment, Selection and Onboarding Policy]* for competency requirements and *[Vetting and DBS Policy]* for DBS procedures.

4. Before Each Activity

Essential Checks (5 minutes)

- [] **Staff present:** Lead + minimum ratio met, all DBS checked
- [] **First aid kit** available and checked
- [] **Emergency contacts - DSL** numbers available
- [] **Venue safe** - quick visual check, emergency exits clear
- [] **Weather** appropriate for activity (outdoor sessions)
- [] **Participant registration** encouraged (mandatory for trips)

Risk Assessment Requirements

- **Annual risk assessment** for all regular activities using BYO template
- **Updated assessment** for any significant changes (new venue, equipment, participants)
- **Dynamic risk assessment** during activities
- **Pre-visit risk assessment** for all offsite activities

See also: **[Health and Safety Policy]** for comprehensive risk assessment guidance and **[Offsite Visits Policy]** for trip-specific requirements.

5. During Activities

Registration and Attendance

- Open Access: Encourage sign-in/sign-out but don't prevent participation if refused
- Closed Sessions/Trips: Mandatory registration required
- Note any departures when observed (but participants free to leave)

Safety Monitoring

- Regular visual checks of participants currently present
- Address safety concerns immediately
- Separate boys/girls for Sports Drive as planned
- **Remember:** Participants can choose to leave - don't prevent departures
- Monitor for signs of bullying, abuse, or concerning behaviour

Behaviour Management

Follow BYO **[Code of Behaviour Policy]** and use positive reinforcement strategies.

Address inappropriate behaviour using graduated response:

1. **Verbal reminder** - explain expected behaviour
2. **Formal warning** - recorded, parents informed if appropriate
3. **Final warning** - may include restricted activities

Report serious incidents to DSL immediately.

Digital Safety

When using technology or online platforms during activities:

- Follow **[Digital Safeguarding Policy]** guidelines
- Monitor internet use appropriately
- Ensure professional communication standards
- Maintain appropriate digital boundaries

See also: *[Digital Safeguarding Policy]* for comprehensive online safety procedures.

6. If Something Goes Wrong

Minor injuries/incidents:

- Give first aid if trained
- Record on simple incident log
- Inform parents at pickup
- Report to **DSL** if **safeguarding** implications

Serious incidents or safeguarding concerns:

- **Immediate action:** Ensure safety of all participants
- Call 999 if medical emergency
- Contact **DSL** immediately: 07597 874 222
- Contact parents immediately (unless **safeguarding** advice suggests otherwise)
- Complete detailed incident report
- Do not investigate allegations yourself

See also: *[Child Protection Procedures]* for detailed incident response and *[Managing Allegations about Staff and Volunteers]* for staff-related concerns.

Missing Participant (Open Access)

- Immediate headcount and area search
- Check attendance register
- Contact participant's emergency contact
- Notify police if **child** cannot be located within 15 minutes
- Inform **DSL/Governor** immediately

7. Record Keeping (Minimal)

Essential Records:

1. Attendance register (simple names/dates list)
2. Incident log (basic details only)
3. Annual risk assessment (using BYO template)
4. Safeguarding concerns (detailed records in secure filing)

Data Protection

All records stored securely following BYO **[Data Protection Policy]**:

- Personal data processed lawfully with appropriate consent
- Information shared only on need-to-know basis
- Photography consent obtained through registration process

See also: *[Data Protection Policy]* for detailed information handling procedures.

8. Activity Specific Notes

Sports Drive

- Boys and girls separate sessions
- Equipment safety checked weekly
- Indoor space - follow **[Health & Safety Policy]** requirements

Allotment Project

- Weather dependent - cancel if unsafe conditions following **[Health & Safety Policy]**
- Tool safety - brief check before use, supervised handling
- Hand washing facilities available
- Environmental risk assessment completed

Homework Club

- Quiet, safe environment
- Monitor internet use following **[Digital Safeguarding Policy]** guidelines
- Support individual needs as able
- One-to-one supervision guidelines followed

9. Trips and Offsite Visits

Note: These are **closed sessions** requiring completed consent forms - not open access.

Additional Requirements

Following **[Offsite Visits Policy]**:

- **Written consent** from parents (mandatory - no implied consent for trips)
- **Enhanced risk assessment** including pre-visit where possible
- **Higher staff ratio** (1:6 minimum)
- **Emergency contact** details for all participants
- **Transport safety** check and appropriate insurance
- **Governor** approval for all trips
- **Registration mandatory** - participants cannot join trips without proper consent

Complex Trips Require:

- Board approval (overseas, overnight, adventurous activities)
- Additional planning time (up to 2 years for complex trips)
- Specialist competency assessment

See also: *[Offsite Visits Policy]* for comprehensive trip planning guidance and *[Health and Safety Policy]* for transport and venue safety requirements.

10. Emergency Contacts

Always have these available:

- **Emergency services:** 999
- **Designated Safeguarding Lead:** Hassan Kingsley (07597 874 222)
- **BYO main contact:** 0121 448 7378
- **Local Authority LADO:** 0121 675 1669
- **University Hospitals Birmingham Trust switchboard:** 0121 554 3801

11. Inclusion and Diversity

Following BYO's **[Equality, Diversity and Inclusion Policy]**:

- Welcome all participants regardless of protected characteristics
- Make reasonable adjustments for disabilities
- Challenge discriminatory behaviour immediately
- Report equality concerns to **DSL**
- Ensure activities are culturally sensitive and inclusive

See also: [Equality, Diversity and Inclusion Policy] and [Organisational Values and Inclusion Framework] for detailed guidance.

12. Complaints and Concerns

- **Complaints about services:** Follow BYO **[Complaints Policy]**
- **Safeguarding concerns:** Report to DSL immediately
- **Staff/volunteer concerns:** Follow BYO **[Whistleblowing Policy]**
- **Participant behaviour:** Follow **[Code of Behaviour Policy]** procedures

See also: [Complaints Policy] and [Whistleblowing Policy] for detailed procedures.



This streamlined procedure balances safety requirements with practical delivery for volunteer-run activities while ensuring full compliance with BYO's policy framework.

Reviewed by:

Hassan Kingsley

Date: 29/06/2026

Next Review Date: 01/07/2027

This policy forms part of BYO's commitment to safeguarding and should be read alongside our complete Safeguarding Framework.

*Britannia Youth Organisation CIC is a Community Interest Company registered in England and Wales.
Company No. 12515346 | Registered Address: 36 St Joseph's Rd, Ward End, Birmingham, B8 2JU*



Basic Attendance Register

USED EVERY SESSION

Session Type: Open Access / Closed Session (circle one)

Activity: _____ Date: _____

Staff Present: _____

For Open Access: Encourage but don't enforce sign-in/sign-out

Participant Name	Age	Arrived	Left	Safeguarding Notes

Emergency evacuation count (if needed): _____

Simple Incident Log



Keep Blank Copies Available

Activity:

Date:

Staff

Type of Incident:

Accident/Behaviour/Safeguarding Concern/Other

What Happened:

Include what led up to it, what occurred, and any relevant medical or behavioural details.

Who was involved:

Include any known health or medical needs relevant to the incident.

Actions Taken:

What immediate actions were taken in response.

E.g.: Administered first aid, Removed child from activity, Contacted parent, Logged on safeguarding system

Follow up Required:

What follow-up is needed (if any).

Could include: A welfare call to family, Monitoring behaviour, Completing a safeguarding referral

First aid given: Yes/No

By Whom:

Parents Informed: Yes/No

By Whom:

DSL Notified: Yes/No

Time:

Serious incident requiring management notification: Yes/No

Emergency Contacts

DSL: Hassan Alexander Kingsley – 07597

Deputy DSL: To be appointed

Emergency: 999

BRITANNIA YOUTH ORGANISATION ACTIVITY RISK ASSESSMENT TEMPLATE

EMERGENCY SAFEGUARDING CONTACTS

DSL: Hassan Kingsley (07597 874 222)

DEPUTY DSL: To be appointed

EMERGENCY SERVICES: 999

ACTIVITY DETAILS

Activity Title: _____

Date & Time: _____

Location and Venue: _____

Lead Staff/Person Responsible: _____

Assessment Completed By: _____

Date of Assessment: _____

ACTIVITY OVERVIEW

Brief Description of Activity (What will participants be doing?):

Activity Type:

☐ Regular Session ☐ Special Event ☐ Offsite Visit ☐ New Activity

PARTICIPANTS AND STAFFING

Number of Expected Participants:

Participant Groups:

☐ Children (Under 18)

☐ Young Adults (18-24)

☐ Adults at Risk

☐ External Visitors

Staff/Volunteer Ratio:

Staff on Duty:

Special Considerations:

☐ Participants with Additional Needs

☐ Staff/volunteer concern

☐ Vulnerable individuals requiring enhanced supervision

☐ Mixed Age Groups requiring adapted approach

COMPREHENSIVE RISK ASSESSMENT					
Hazard/Risk	Who Might Be Harmed?	Risk Level (L/M/H)	Control Measures	Residual Risk (L/M/H)	Review Date
Physical Safety Risks: Slips, trips, falls					
Equipment/Activity Risks: Equipment injuries, activity-specific hazards					
Environmental Risks: Weather, venue-specific hazards					
Safeguarding Risks: Participant vulnerability, supervision concerns					
Behavioural Risks: Challenging behaviour, peer conflicts					
Emergency Response: Access to help, communication					
Additional Risk:					
Additional Risk:					

SAFEGUARDING CONSIDERATIONS

Specific Safeguarding Risks Identified:

Additional Safeguarding Measures:

- ☐ Enhanced supervision ratios
 ☐ Specific staff deployment
 ☐ Individual support plans
☐ Environmental modifications
 ☐ Communication with parents/carers
 ☐ Coordination with external agencies

Staff Safeguarding Competence Confirmed:

- ☐ All staff have current DBS checks
 ☐ All Staff have completed safeguarding training
☐ Staff understand reporting procedures
 ☐ DSL contact details available to all staff

THIRD PARTY DOCUMENTATION

(Complete if venue or activity involves external providers)

Document Requested	Received?	Notes/Reference Numbers
Venue/Provider Risk Assessment	☐ Yes ☐ No ☐ N/A	
Public Liability Insurance Certificate	☐ Yes ☐ No ☐ N/A	
Staff Qualifications/Licenses	☐ Yes ☐ No ☐ N/A	
Safeguarding Policy/DBS Details	☐ Yes ☐ No ☐ N/A	
Emergency Procedures	☐ Yes ☐ No ☐ N/A	
Activity-Specific Certifications	☐ Yes ☐ No ☐ N/A	

EMERGENCY PROCEDURES

Nearest First Aider: _____

Location of First Aid: _____

Fire Exit/Evacuation Plan: _____

Mobile Reception on Site: ☐ Yes ☐ No

Nearest Hospital: _____

Emergency Contact Numbers Available:

☐ Emergency Services: 999

☐ DSL: 07597 874 222

☐ BYO Office: 0121 448 7378

☐ Venue Emergency Contact

Emergency Response Procedures:

POLICY COMPLIANCE CHECKLIST

☐ Health and Safety Policy – Physical safety measures addressed

☐ Safeguarding Policy Framework – Safeguarding Risks Addressed

☐ Child Protection Procedures –

☐ Code of Conduct Policy – Professional standards maintained

☐ Equity, Diversity, and Inclusion Policy - Accessibility Considered

☐ Data Protection Policy – Information Handling appropriate

Additional Policy Considerations:

☐ Digital Safeguarding Policy - if technology used ☐ Anti-bullying Policy - Prevention Measures in Place

☐ Offsite Visits Policy - if activity off-premises ☐ Managing Allegations Procedures - Staff aware of protocols

REVIEW AND MONITORING

Next Review Date: _____

Trigger for Review:

- ☐ Change in venue or equipment
- ☐ Incident or near miss
- ☐ Change in Participant group
- ☐ New staff or volunteers
- ☐ Change in External Providers
- ☐ Feedback Indicating Concerns

Additional Notes:

This risk assessment should be read alongside

Safeguarding Policy Framework • Health and Safety Policy • Child Protection Procedures • Code of Conduct Policy • Emergency procedures and contact details available to all staff

APPROVAL AND SIGN-OFF

Name	Role	Signature	Date
Completed By:			
Reviewed By:			
DSL Approval:	DSL		

- ☐ Risk Assessment approved - activity may proceed
- ☐ Risk assessment requires amendments before approval

Comments:



Trip Consent Form

Britannia Youth Organisation CIC

Please complete all sections clearly and return before the trip departure date

Trip Information

Trip:

Date:

dd/mm/yyyy



Time:

 :

Return time:

 :

Destination:

Transport:

Participant Details

Participant name:

Age:

Date of Birth:

Address:

Emergency Contacts

Primary Contact

Name:

Relationship:

Phone:

Alternative Phone:

Secondary Contact

Name:

Relationship:

Phone:

Alternative Phone:

Medical Information

Medical Conditions/Allergies

Please list any medical conditions, allergies, or health concerns

Medical Conditions/Allergies

List any medications the participant is currently taking

Medical Conditions/Allergies

Any dietary restrictions, allergies, or special requirements

Swimming ability

can swim, can't swim, weak swimmer

GP Name and Practice:

Additional Information

Special support needs:

Any additional support, accessibility needs, or considerations

Emergency contact restrictions

Any restrictions on who can be contacted or information shared

Communication needs:

Any restrictions on who can be contacted or information shared

Consent

I give permission for my child to attend this trip and understand that:

- ☐ BYO will supervise appropriately following our safeguarding and safety policies
- ☐ Emergency contacts will be used if needed
- ☐ Medical treatment may be sought if required in an emergency
- ☐ Staff will take reasonable care but cannot guarantee against injury or accident
- ☐ Photography may be taken as per BYO's Photography and Videography Policy
- ☐ My child may be excluded from activities if behaviour becomes unsafe
- ☐ Transport arrangements are as detailed above
- ☐ Information will be handled according to BYO's Data Protection Policy
- ☐ Safeguarding procedures will be followed if concerns arise

Parent/Guardian

Parent/Guardian Signature:

sign here

Date:

dd/mm/yyyy



Print Name:

Relationship to child:

For BYO Use Only

☐

Risk assessment completed

☐

Staff ratio met

☐

All consents collected

☐

Emergency contacts available

☐

Safeguarding considerations reviewed

☐

Insurance confirmed

Trip Leader:

Date:

dd/mm/yyyy



Comprehensive Trip Risk Assessment

Britannia Youth Organisation CIC

Complete this assessment thoroughly before every offsite visit - safeguarding and safety depend on it

Trip/Activity Overview

Trip/Activity:

Date:

dd/mm/yyyy



Transport:

Trip Leader:

Number of Participants:

Ages:

E.g. 12-18

Number of Staff:



Transport Risks

☐

Vehicle roadworthy and insured

☐

Driver qualified and checked

☐

Journey planned

☐

Safeguarding considerations for transport arrangements

Transport Notes:

Detail any specific arrangement, risk identified, or additional measures taken....



Venue Risks

☐ Venue suitable for group

☐ Emergency exits identified

☐ First aid facilities available

☐ Safeguarding policies of venue checked

☐ Venue staff background checks confirmed

Venue Notes:

Detail venue-specific risks, accessibility considerations, safeguarding arrangements



Activity Risk

☐ Equipment safe and appropriate

☐ Supervision adequate

☐ Participants briefed

☐ Instructor qualifications checked

☐ Safeguarding considerations for activities

Activity Notes:

Describe specific activities, equipment used, supervision arrangements, and any special considerations...



Participant Risks

☐ Medical needs identified

☐ Behaviour plans in place

☐ Emergency contacts available

☐ Individual safeguarding considerations reviewed

☐ Support plans in place

Participant Notes:

Detail any specific participant needs, vulnerabilities, medical requirements, or support arrangements...



Environmental Risks

☐ Weather appropriate

☐ Backup plan available

☐ Appropriate clothing advised

☐ Safeguarding implications of environment considered

☐ Support plans in place

Environmental Notes:

Weather conditions, backup plans, environment hazards, clothing requirements...

Safeguarding Risk Assessment

Specific safeguarding considerations:

Risk:

Describe the safeguarding risk...

How Managed:

Describe how the safeguarding risk will be managed...

Risk:

Describe the safeguarding risk...

How Managed:

Describe how the safeguarding risk will be managed...

Risk:

Describe the safeguarding risk...

How Managed:

Describe how the safeguarding risk will be managed...



Emergency Procedures

☐ Emergency contacts available

☐ First aid kit taken

☐ Mobile phone charged

☐ Staff know emergency procedures

☐ Venue emergency procedures understood

☐ Safeguarding incident procedures understood

☐ DSL contact details available

Assessment Approval

Assessment completed by:

Date:

dd/mm/yyyy



Approved by DSL:

sign here

Date:

dd/mm/yyyy



Document Control:

- This assessment balances safety and safeguarding requirements with practical delivery
 - Focus on comprehensive planning while keeping young people safe
 - Regular review ensures continued effectiveness and compliance

Britannia Youth Organisation CIC

Community Interest Company registered in England and Wales
Company No. 12515346 | Registered Address: 36 St Joseph's Rd, Ward End, Birmingham, B8 2JU



Pre-Activity Safety Checklist

Please complete all sections clearly and return before the trip departure date

Trip Information

Trip:

Date:

dd/mm/yyyy



Time:

 :

Return time:

 :

Destination:

Transport:

Participant Details

Participant name:

Age:

Date of Birth:

Address:

Emergency Contacts

Primary Contact

Name:

Relationship:

Phone:

Alternative Phone:

Secondary Contact

Name:

Relationship:

Phone:

Alternative Phone:

Medical Information

Medical Conditions/Allergies

Please list any medical conditions, allergies, or health concerns

Medical Conditions/Allergies

List any medications the participant is current taking

Medical Conditions/Allergies

Any dietary restrictions, allergies, or special requirements

Swimming ability

can swim, can't swim, weak swimmer

GP Name and Practice:

Additional Information

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Any restrictions on who can be contacted or information shared

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- ☐ My child may be excluded from activities if behaviour becomes unsafe
- ☐ Transport arrangements are as detailed above
- ☐ Information will be handled according to BYO's Data Protection Policy
- ☐ Safeguarding procedures will be followed if concerns arise

Parent/Guardian

Parent/Guardian Signature:

sign here

Date:

dd/mm/yyyy



Print Name:

Relationship to child:

For BYO Use Only

☐

Risk assessment completed

☐

Staff ratio met

☐

All consents collected

☐

Emergency contacts available

☐

Safeguarding considerations reviewed

☐

Insurance confirmed

Trip Leader:

Date:

dd/mm/yyyy

BRITANNIA YOUTH ORGANISATION SAFEGUARDING CASE REPORT FROM

Confidential – Handle in accordance with the Data Protection Policy

URGENT – Complete Immediately for all Safeguarding Concerns

If immediate danger: Call 999 first, then complete this form

Submit to DSL within 24 hours: Hassan Kingsley (07597 874 222)

SECTION 1: INCIDENT DETAILS

Date of incident: _____ Time: _____

Date of report: _____ Time: _____

Location of Incident: _____

Activity/Session: _____

SECTION 2: REPORTING PERSON

Name: _____ Role: _____

Contact Number: _____ Email: _____

SECTION 3: CHILD/YOUNG PERSON DETAILS

Full name: _____ Age: _____

Date of birth: _____ Gender: _____

Address (if known):

Parent/Carer 1: _____ Contact: _____

Parent/Carer 2: _____ Contact: _____

Child/Young Person Known Status

- ☐ Regular BYO attendee ☐ First time at BYO
- ☐ Occasional attendee ☐ Known to other services (specify below)

Other services involved:

SECTION 4: TYPE OF CONCERN

Primary Concern Type (tick all that apply)

- | | |
|--|--|
| ☐ Physical abuse | ☐ Bullying/peer abuse |
| ☐ Sexual abuse | ☐ Radicalisation |
| ☐ Emotional abuse | ☐ County lines/exploitation |
| ☐ Neglect | ☐ Self-harm |
| ☐ Domestic violence | ☐ Mental health crisis |
| ☐ Child sexual exploitation | ☐ Staff/volunteer concern |
| ☐ Online/digital abuse | ☐ Other: <input type="text"/> |

SECTION 5: DETAILED INCIDENT DESCRIPTION

What happened? (Use child's own words where possible, be factual, avoid opinions)

Who was present/involved?

How did the concern come to light? (disclosure, observation, report from others)

SECTION 6: CHILD’S PRESENTATION AND RESPONSE

Child’s emotional state and behaviour:

Any injuries or physical signs observed:

Child’s own explanation/account:

SECTION 7: IMMEDIATE ACTION TAKEN

Actions Taken (tick all that apply)

- ☐ First aid administered

☐ Medical attention sought (999/hospital)

☐ Police contacted (999/101)

☐ DSL/DDSL notified immediately

☐ Parents/carers contacted

☐ Child made safe/removed from situation

☐ Witnesses separated
- ☐ Emotional support provided to child

☐ Other children safeguarded

☐ Evidence preserved

☐ Incident area secured

☐ Photos taken (with appropriate consent)

☐ Other:

Details of immediate actions and timings:

SECTION 8: NOTIFICATIONS MADE

Person/Agency Contacted	Time/Date	Method	Response/Action
DSL/DDSL:		Phone/Email/In person	
Emergency Services:		999/101	
Parent/Carer:		Phone/In person	
Children's Services:		Phone/Email	
Other:			

SECTION 9: PREVIOUS CONCERNS

Any previous concerns about this child/family?

- ☐ No previous concerns known ☐ Previous welfare notes ☐ Previous safeguarding concerns

SECTION 10: RISK ASSESSMENT

Current Risk Level

- ☐ **Low risk** - monitoring and support adequate
- ☐ **Medium risk** - enhanced support and monitoring needed
- ☐ **High risk** - immediate external intervention required
- ☐ **Immediate danger** - emergency services involvement

Risk factors and vulnerabilities:

Protective factors and support:

SECTION 11: RECOMMENDATIONS AND NEXT STEPS

Recommended actions:

Support needed for child/family:

Follow-up arrangements:

SECTION 12: FORM COMPLETION

Completed by: _____

Signature: _____

Date completed: _____

Time: _____

FOR DSL/DDSL USE ONLY

Received by: _____

Date/Time: _____

DSL Assessment and Action Plan:

Actions Taken by DSL

- ☐ Referred to Children's Services (MASH)
- ☐ Police referral made
- ☐ LADO contacted
- ☐ Early Help referral
- ☐ Internal investigation initiated
- ☐ No further action at this time
- ☐ Monitoring and review arranged
- ☐ Other:

Case Reference Number:

DSL Signature: _____

Date: _____

QUICK WELFARE NOTE

For welfare concerns beyond normal poverty/play - Takes 2 minutes

NOT for: Worn shoes, play-dirty clothes, food stains, general poverty signs, asking for snacks (we always have them!)

USE for: Appears malnourished/very thin, seems unwell/upset, mentions worrying things about home, unusual behaviour changes, injuries

Date: _____ **Child:** _____ **Staff:** _____

- | | | |
|---|---|--|
| ☐ Appears malnourished/emaciated | ☐ Seems unwell/tired | ☐ Upset/anxious/withdrawn |
| ☐ Mentioned home worries | ☐ Injuries/marks | ☐ Other concern |

What made you concerned (not just appearance):

Action taken: ☐ Had a chat ☐ Gave extra support ☐ Just kept an eye on them

Next Steps: ☐ Continue monitoring ☐ Mention to DSL ☐ Needs proper safeguarding

DSL Told: Date/Time **By:** _____

Remember: Poverty ≠ neglect. Focus on child's well-being changes, not their economic situation | **DSL:** Hassan Kingsley
07597 874 222 | **Emergency:** 999

CUT HERE

QUICK WELFARE NOTE

For welfare concerns beyond normal poverty/play - Takes 2 minutes

NOT for: Worn shoes, play-dirty clothes, food stains, general poverty signs, asking for snacks (we always have them!)

USE for: Appears malnourished/very thin, seems unwell/upset, mentions worrying things about home, unusual behaviour changes, injuries

Date: _____ **Child:** _____ **Staff:** _____

- | | | |
|---|---|--|
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| ☐ Mentioned home worries | ☐ Injuries/marks | ☐ Other concern |

What made you concerned (not just appearance):

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07597 874 222 | **Emergency:** 999